HIGHLIGHTS OF PRESCRIBING INFORMATION
These highlights do not include all the information needed to use MYQORZO safely and effectively. See full prescribing information for MYQORZO.

MYQORZO™ (aficamten) tablets, for oral use
Initial U.S. Approval: 2025

WARNING: RISK OF HEART FAILURE
See full prescribing information for complete boxed warning.

- **MYQORZO can cause heart failure due to systolic dysfunction. (5.1)**
- **Echocardiogram assessments of left ventricular ejection fraction (LVEF) are required before and during MYQORZO use. (2.1)**
- **Initiation in patients with left ventricular ejection fraction (LVEF) <55% is not recommended. (2.1)**
- **Decrease dose if LVEF <50% and ≥40%. Interrupt dosing if LVEF <40% or if worsening clinical status. (2.2)**
- **MYQORZO is available only through a restricted program called the MYQORZO REMS Program. (5.2)**

----------INDICATIONS AND USAGE----------

MYQORZO is a cardiac myosin inhibitor indicated for the treatment of adults with symptomatic obstructive hypertrophic cardiomyopathy (oHCM) to improve functional capacity and symptoms. (1)

----------DOSAGE AND ADMINISTRATION----------

- Recommended starting dose is 5 mg orally once daily. (2.1)
- Dosage is individualized based on echocardiographic assessments and clinical status. Refer to Full Prescribing Information for instructions on dosage modification. (2.1, 2.2)

----------DOSAGE FORMS AND STRENGTHS----------

Film-coated tablets: 5 mg, 10 mg, 15 mg, 20 mg. (3)

----------CONTRAINDICATIONS----------

- Rifampin (4)

----------WARNINGS AND PRECAUTIONS----------

Risk of Heart Failure: Consider decreasing dose or dose interruption in patients with serious intercurrent illness. (2.2, 5.1)
Drug Interactions Leading to Increased Risk of Heart Failure or Loss of Effectiveness: Advise patients of the potential for drug interactions. (2.3, 5.3, 17)

----------ADVERSE REACTIONS----------

The most common adverse reaction was hypertension (8%). (6.1)

To report SUSPECTED ADVERSE REACTIONS, contact Cytokinetics at 1-833-633-2986 or FDA at 1-800-FDA-1088 or www.fda.gov/medwatch.

----------DRUG INTERACTIONS----------

Drugs that inhibit multiple pathways of MYQORZO elimination, strong CYP2C9 inhibitors, or moderate-to-strong CYP3A inducers may increase risk of heart failure. MYQORZO dose reduction and additional monitoring may be required when initiating or discontinuing these drugs. (2.3, 7.1)

See 17 for PATIENT COUNSELING INFORMATION and FDA-approved Medication Guide.

Revised: 12/2025

FULL PRESCRIBING INFORMATION: CONTENTS*

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FULL PRESCRIBING INFORMATION

WARNING: RISK OF HEART FAILURE

MYQORZO reduces left ventricular ejection fraction (LVEF) and can cause heart failure due to systolic dysfunction *[see Warnings and Precautions (5.1)]*.

Echocardiogram assessments are required prior to and during treatment with MYQORZO to monitor for systolic dysfunction. Initiation of MYQORZO in patients with LVEF <55% is not recommended. Decrease the dose of MYQORZO if LVEF is <50% and ≥40% *[see Dosage and Administration (2.2) and Warnings and Precautions (5.1)]*. Interrupt the dose of MYQORZO if LVEF <40% or if the patient experiences heart failure symptoms or worsening clinical status due to systolic dysfunction *[see Dosage and Administration (2.2)]*.

Because of the risk of heart failure due to systolic dysfunction, MYQORZO is available only through a restricted program under a Risk Evaluation and Mitigation Strategy (REMS) called the MYQORZO REMS Program *[see Warnings and Precautions (5.2)]*.

1 INDICATIONS AND USAGE

MYQORZO is indicated for the treatment of adults with symptomatic obstructive hypertrophic cardiomyopathy (oHCM) to improve functional capacity and symptoms.

2 DOSAGE AND ADMINISTRATION

2.1 Evaluation Before and During Use of MYQORZO

Initiation or up-titration of MYQORZO in patients with LVEF <55% is not recommended.

Patients may develop heart failure while taking MYQORZO. Regular LVEF and Valsalva left ventricular outflow tract gradient (LVOT-G) assessment is needed for titration to achieve an appropriate target Valsalva LVOT-G, while maintaining LVEF ≥50% and avoiding heart failure symptoms.

2.2 Recommended Dosage and Administration

The recommended starting dose of MYQORZO is 5 mg orally once daily. Increase the dose every 2 to 8 weeks by 5 mg until a maintenance dose or the maximum recommended dose of 20 mg once daily is achieved. The maintenance dose of MYQORZO is individualized based on the patient's LVEF and LVOT-G. Recommendations for dosing based on LVEF and LVOT-G criteria are provided in Table 1.

Table 1: Dose Adjustment of MYQORZO

LVEF	Valsalva LVOT-G	Dose Adjustment
≥55%	≥30 mmHg	Increase dose by 5 mg (up to the maximum dose of 20 mg once daily)
≥55%	<30 mmHg	Maintain Dose
<55% and ≥50%	Any	Maintain Dose
<50% and ≥40%	Any	Decrease dose by 5 mg[1] If already on 5 mg, interrupt treatment for at least 7 days
<40%	Any	Interrupt treatment for at least 7 days

1. Decrease dose as follows: 20 mg to 15 mg; 15 mg to 10 mg; 10 mg to 5 mg

Perform an echocardiographic assessment 2 to 8 weeks after initiation of treatment or any dose adjustment (e.g., due to LVEF and LVOT-G criteria or drug interaction). After a treatment interruption due to low LVEF, resume treatment, no earlier than 7 days, when LVEF ≥55% and re-initiate dose titration at the starting dose of 5 mg (see Table 1).

After the maintenance dose has been established, assess LVEF and Valsalva LVOT-G every 6 months, or every 3 months in patients with LVEF <55% to ≥50%. Consider monitoring LVEF and adjust the dose per Table 1 as needed, in patients with an intercurrent illness (e.g., severe infection or COVID-19), new arrhythmia (e.g., new or uncontrolled atrial fibrillation or other uncontrolled tachyarrhythmia) or any other conditions that may impair systolic function. Do not increase the dose until the intercurrent illness or new arrhythmia has resolved or stabilized.

MYQORZO should be taken once daily with or without meals at about the same time every day. Swallow tablets whole.

2.3 Dosage Modifications for Drug Interactions

Initiate MYQORZO at the recommended starting dose of 5 mg once daily in patients who are on stable therapy with fluconazole, voriconazole, fluvoxamine, strong CYP2C9 inhibitors, or in patients discontinuing a moderate to strong CYP3A inducer.

Concomitant Administration with Fluconazole or Voriconazole

In patients who initiate fluconazole (if used for more than 3 days) or voriconazole, reduce the dose of MYQORZO to 5 mg if they are currently receiving 15 mg or 20 mg. Avoid concomitant use if patients are currently receiving MYQORZO 5 mg or 10 mg. Assess LVEF and LVOT-G 2 to 8 weeks after initiation of fluconazole or voriconazole and titrate the dose of MYQORZO according to Table 1 *[see Dosage and Administration (2.2), Warnings and Precautions (5.3) and Drug Interactions (7.1)]*.

Concomitant Administration with Fluvoxamine or Strong CYP2C9 Inhibitors

In patients who initiate fluvoxamine or a strong CYP2C9 inhibitor, reduce the dose of MYQORZO (20 mg to 10 mg; 15 mg to 5 mg; 10 mg to 5 mg). For patients currently receiving MYQORZO 5 mg, maintain the 5 mg dose. Assess LVEF and LVOT-G 2 to 8 weeks after inhibitor initiation and titrate

the dose of MYQORZO according to Table 1 *[see Dosage and Administration (2.2), Warnings and Precautions (5.3) and Drug Interactions (7.1)]*.

Concomitant Administration with Moderate to Strong CYP3A Inducers
In patients who are on stable therapy with moderate to strong CYP3A inducers (e.g., carbamazepine), when discontinuing these medications, reduce the dose of MYQORZO (20 mg to 10 mg; 15 mg to 5 mg; 10 mg to 5 mg). For patients currently receiving MYQORZO 5 mg, maintain the 5 mg dose. Assess LVEF and LVOT-G 2 to 8 weeks after inducer discontinuation and titrate the dose of MYQORZO according to Table 1 *[see Dosage and Administration (2.2), Warnings and Precautions (5.3) and Drug Interactions (7.1)].*

2.4 Missed Dose

If a dose is missed, it should be taken as soon as possible on the same day. The next scheduled dose should be taken at the usual time the following day. Two doses of MYQORZO should not be taken on the same day.

3 DOSAGE FORMS AND STRENGTHS

MYQORZO is available as tablets in the following strengths:

Tablets: 5 mg - purple, round tablet debossed with “5” on one side and “CK” on the other side.

Tablets: 10 mg - purple, triangular tablet debossed with “10” on one side and “CK” on the other side.

Tablets: 15 mg - purple, pentagonal tablet debossed with “15” on one side and “CK” on the other side.

Tablets: 20 mg - purple, oval tablet debossed with “20” on one side and “CK” on the other side.

4 CONTRAINDICATIONS

MYQORZO is contraindicated with concomitant use of rifampin *[see Warnings and Precautions (5.3) and Drug Interactions (7.1)]*.

5 WARNINGS AND PRECAUTIONS

5.1 Heart Failure

MYQORZO reduces cardiac contractility, which can reduce LVEF and cause heart failure.

Patients who experience a serious intercurrent illness (e.g., serious infection) or arrhythmia (e.g., new or uncontrolled atrial fibrillation) may be at greater risk of developing systolic dysfunction and heart failure *[see Clinical Trial Experience (6.1)]*. Asymptomatic LVEF reduction, intercurrent illnesses, and arrhythmias require additional monitoring considerations *[see Dosage and Administration (2.2)].*

Assess the patient’s clinical status and LVEF prior to and regularly during treatment and adjust the MYQORZO dose accordingly *[see Dosage and Administration (2.3)].* New or worsening arrhythmia, dyspnea, chest pain, fatigue, leg edema, or elevations in N-terminal pro-B-type natriuretic peptide (NT-proBNP) may be signs and symptoms of heart failure and should prompt an evaluation of cardiac function.

Initiation of MYQORZO in patients with LVEF <55% is not recommended.

5.2 MYQORZO REMS Program

MYQORZO is available only through a restricted program called the MYQORZO REMS Program, because of the risk of heart failure due to systolic dysfunction *[see Warnings and Precautions (5.1)].*

Notable requirements of the MYQORZO REMS Program include the following:

- Prescribers must be certified by enrolling in the MYQORZO REMS Program.
- Patients must enroll in the MYQORZO REMS Program and comply with ongoing monitoring requirements *[see Dosage and Administration (2.1)]*.
- Pharmacies must be certified by enrolling in the MYQORZO REMS Program and must only dispense to patients who are authorized to receive MYQORZO.
- Wholesalers and distributors must only distribute to certified pharmacies.

Further information is available at www.MYQORZOREMS.com or by telephone at 1-844-285-7367.

5.3 Cytochrome P450 Interactions Leading to Heart Failure or Loss of Effectiveness

MYQORZO is metabolized primarily by CYP2C9, and to a lesser extent by CYP3A, CYP2D6, and CYP2C19 enzymes. Initiation of medications that inhibit multiple P450 pathways of MYQORZO elimination (e.g., fluconazole, voriconazole, fluvoxamine) or strong CYP2C9 inhibitors, and discontinuation of moderate-to-strong CYP3A inducers may lead to increased blood concentrations of aficamten and increase the risk of heart failure due to systolic dysfunction *[see Contraindications (4), Warnings and Precautions (5.1), and Drug Interactions (7.1)]*. Conversely, initiation of medications that induce P450 pathways of MYQORZO (e.g., rifampin, moderate-to-strong CYP3A inducers) may lead to decreased blood concentrations of aficamten and potential loss of effectiveness *[see Contraindications (4) and Drug Interactions (7.1)]*. Assess LVEF 2 to 8 weeks after initiation of such inhibitors or after discontinuation of such inducers and adjust the dose of MYQORZO accordingly *[see Dosage and Administration (2.3)]*.

Advise patients of the potential for drug interactions. Advise patients to inform their healthcare provider of all concomitant medications prior to and during MYQORZO treatment *[see Drug Interactions (7.1) and Patient Counseling Information (17)]*.

6 ADVERSE REACTIONS

The following adverse reaction is discussed in other sections of labeling:

- Heart Failure *[see Warnings and Precautions (5.1)]*

6.1 Clinical Trials Experience

Because clinical trials are conducted under widely varying conditions, adverse reaction rates observed in the clinical trials of a drug cannot be directly compared to rates in the clinical trials of another drug and may not reflect the rates observed in practice.

The safety of MYQORZO was evaluated in SEQUOIA-HCM, a phase 3, randomized, double-blind, placebo-controlled study *[see Clinical Studies (14)]*. Of the 282 adults with oHCM, 142 patients received daily doses of MYQORZO (initiated at 5 mg and titrated up to a maximum dose of 20 mg) and 140 patients received placebo. The median treatment duration for patients receiving MYQORZO was ~24 weeks (range 4 to 29 weeks).

Hypertension (8% vs. 2%) was the only adverse reaction occurring in >5% of patients and more commonly on MYQORZO than on placebo.

Eligible oHCM patients were able to participate in an ongoing, open-label, single-arm, long-term safety study (FOREST-HCM). Based on available data, the safety profile of MYQORZO in FOREST-HCM was similar to that observed in SEQUOIA-HCM.

Effects on Systolic Function

In SEQUOIA-HCM, the mean (SD) resting LVEF at baseline was 75% (6) in both treatment groups. Consistent with the mechanism of action of MYQORZO, LS mean (SE) change from baseline in LVEF

was -7% (0.6) in the MYQORZO group and -2% (0.6) in the placebo group at the end of the 24-week treatment period. Four weeks after the end of treatment, mean LVEF was similar between the MYQORZO and placebo groups. During the 24-week treatment period, 5 (4%) patients in the MYQORZO group and 1 (1%) patient in the placebo group experienced a reversible reduction in LVEF to <50% (median LVEF: 47%; range 34% - 49% for these 5 patients in the MYQORZO group). Reductions in LVEF to <50% did not require treatment interruption and were not associated with clinical heart failure *[see Warnings and Precautions (5.1)]*.

Effects on Blood Pressure

In SEQUOIA-HCM, the mean (SD) systolic/diastolic blood pressure (SBP/DBP) at baseline was 125(16)/75(11) mmHg for patients in the MYQORZO group and 126(16)/74(11) mmHg in the placebo group. There was a greater mean change from baseline in SBP/DBP (SD) in the MYQORZO group compared to the placebo group at the end of the 24-week treatment period [2(13)/3(8) mmHg and -3(14)/-1(9) mmHg, respectively]. Four weeks after the end of treatment, the mean SBP/DBP was similar between the MYQORZO and placebo groups. In SEQUOIA-HCM, SBP ≥160 mmHg was observed in approximately 16% of patients in the MYQORZO group and 8% of patients in the placebo group. MYQORZO-associated increases in blood pressure are consistent with relief of LVOT obstruction and improved cardiac output.

7 DRUG INTERACTIONS

7.1. Potential for Other Drugs to Affect Plasma Concentrations of MYQORZO

Aficamten is primarily metabolized by CYP2C9 and, to a lesser extent by CYP3A, CYP2D6, and CYP2C19. Concomitant administration of drugs that inhibit multiple P450 pathways of aficamten elimination, strong inhibitors of CYP2C9, and moderate-to-strong inducers of CYP3A, may affect the exposure of aficamten *[see Warnings and Precautions (5.2) and Clinical Pharmacology (12.3)]* (see Table 2).

Table 2: Established and Potentially Significant Pharmacokinetic Drug-Drug Interactions with MYQORZO

Inhibitors of Multiple CYPs		
Fluvoxamine	Clinical Impact	Fluvoxamine is a strong CYP2C19 inhibitor, weak to moderate CYP3A inhibitor, weak CYP2C9 inhibitor, and weak CYP2D6 inhibitor. Coadministration of fluvoxamine is predicted to increase aficamten exposure, which may increase the risk of developing heart failure due to systolic dysfunction.
	Prevention or Management	Initiate MYQORZO at 5 mg in patients on stable therapy with fluvoxamine. In patients who are on MYQORZO treatment and intend to initiate fluvoxamine: Reduce dose of MYQORZO (i.e., 20 mg to 10 mg, 15 mg to 5 mg, or 10 mg to 5 mg) or continue 5 mg *[see Dosage and Administration (2.3)]*.

Fluconazole or Voriconazole	Clinical Impact	Fluconazole is a moderate CYP2C9 inhibitor, moderate CYP3A inhibitor, and strong CYP2C19 inhibitor. Voriconazole is a strong CYP3A inhibitor, moderate CYP2C19 inhibitor, and weak CYP2C9 inhibitor. Coadministration with fluconazole is observed, and voriconazole is predicted, to increase aficamten exposure, which may increase the risk of developing heart failure due to systolic dysfunction *[see Clinical Pharmacology (12.3)]*.
	Prevention or Management	Initiate MYQORZO at 5 mg in patients on stable therapy with fluconazole or voriconazole. In patients currently receiving MYQORZO who intend to initiate fluconazole (if used more than 3 days) or voriconazole: Reduce dose of MYQORZO to 5 mg if they are currently receiving MYQORZO 15 mg or 20 mg. Avoid concomitant use if currently receiving MYQORZO 5 mg or 10 mg *[see Dosage and Administration (2.3)]* .
Strong CYP2C9 inhibitors		
Clinical Impact		Coadministration with a strong CYP2C9 inhibitor is predicted to increase the exposure of aficamten, which may increase the risk of developing heart failure due to systolic dysfunction *[see Clinical Pharmacology (12.3)]*.
Prevention or Management		Initiate MYQORZO at 5 mg in patients who are on stable therapy with a strong CYP2C9 inhibitor. In patients who are on MYQORZO treatment and intend to initiate a strong CYP2C9 inhibitor: Reduce dose of MYQORZO (i.e., 20 mg to 10 mg, 15 mg to 5 mg, or 10 mg to 5 mg) or continue 5 mg *[see Dosage and administration (2.3)]*.
CYP Inducers		
Rifampin	Clinical Impact	Rifampin is a strong CYP3A inducer, strong CYP2C19 inducer, and moderate CYP2C9 inducer. Coadministration of rifampin decreases aficamten exposure, which may reduce the effectiveness of MYQORZO *[see Contraindications (4) and Clinical Pharmacology (12.3)]*.
	Prevention of Management	Concomitant use with rifampin is contraindicated.
Other moderate-to-strong CYP3A inducers	Clinical Impact	Concomitant use with a moderate to strong CYP3A inducer decreases aficamten exposure, which may reduce MYQORZO effectiveness *[see Clinical Pharmacology (12.3)]*. The risk of heart failure due to systolic dysfunction may increase with discontinuation of these inducers.

	Prevention or Management	In patients who intend to discontinue a moderate to strong CYP3A Inducer: Reduce dose of MYQORZO (20 mg to 10 mg; 15 mg to 5 mg; 10 mg to 5 mg) or continue 5 mg *[see Dosage and Administration (2.3)].*

8 USE IN SPECIFIC POPULATIONS

8.1 Pregnancy

Risk Summary

There are no available data on the use of MYQORZO during pregnancy to evaluate for a drug-associated risk of major birth defects, miscarriage, or other adverse maternal or fetal outcomes. The underlying maternal condition during pregnancy poses a risk to the mother and fetus *(see Clinical Considerations)*. In embryo-fetal and pre- and postnatal development studies, when pregnant rats were administered aficamten during the period of organogenesis, aficamten increased the incidence of structural malformations at exposures ≥4-times the maximum recommended human dose (MRHD) of 20 mg based on free area under the concentration curve (AUC). No adverse effects on development were seen at 3-times the MRHD exposure *(see Data)*.

The background risk of major birth defects and miscarriage for the indicated population is unknown. All pregnancies have a background risk of birth defects, loss, and other adverse outcomes. In the U.S. general population, the estimated background risk of major birth defects and miscarriage in clinically recognized pregnancies is 2% to 4% and 15% to 20%, respectively.

There is a pregnancy safety study for MYQORZO. If MYQORZO is administered during pregnancy, or if a patient becomes pregnant while receiving MYQORZO or within 3 weeks after the last dose of MYQORZO, healthcare providers should report MYQORZO exposure by contacting Cytokinetics, Inc. at 1-833-633-2986.

Clinical Considerations

Disease-Associated Maternal and Embryo-Fetal Risk

Obstructive HCM in pregnancy has been associated with increased risk for preterm birth.

Data

Animal Data

Aficamten given to pregnant rats (2, 6 and 9 mg/kg/day) during the period of organogenesis was associated with increased external fetal malformations (kinked tail) at aficamten exposures 5-times the clinical exposures at the MRHD based on free AUC, with uncertain human relevance. Increased post-implantation loss (early and late resorptions) and decreased mean fetal body weight occurred in the presence of significant maternal toxicity at exposures 5-times the clinical exposures at the MRHD. No adverse effects on embryofetal development were observed at 6 mg/kg/day, representing 3-times the clinical exposure at the MRHD.

Pregnant rabbits given aficamten (5, 10 and 20/15 mg/kg/day) during the period of embryofetal organogenesis showed a numerical increase in post-implantation loss (late resorptions) and fetuses with cardiac malformations at 10 mg/kg/day. However, no increase in malformations was observed at the high dose of 20/15 mg/kg/day, a dose that caused significant maternal toxicity (leading to dose reduction from 20 to 15 mg/kg/day during the study) at clinically relevant exposures.

Pregnant rats given aficamten (0.5, 1.5, 6 mg/kg/day) during embryofetal organogenesis through offspring weaning in a pre- and postnatal development study had reduced offspring viability at the 6 mg/kg/day in the presence of maternal toxicity, at 4-times the clinical exposure at the MRHD based

on free AUC. An increased number of offspring with bent tails was also observed at the 6 mg/kg/day. No adverse maternal toxicity or developmental effects occurred at 1.5 mg/kg/day, which corresponds to clinical exposure at the MRHD. Aficamten did not affect neurobehavioral parameters, sexual maturation, or reproductive function of the offspring at any dose.

8.2 Lactation

Risk Summary

There are no data on the presence of aficamten in human milk, the effects on the breastfed infant, or the effects on milk production. Aficamten was detected in the plasma of rat pups when dams were dosed with the drug orally during the lactation period. When a drug is present in animal milk, it is likely that the drug will be present in human milk. The developmental and health benefits of breastfeeding should be considered along with the mother's clinical need for MYQORZO and any potential adverse effects on the breastfed infant from MYQORZO or from the underlying maternal condition.

8.4 Pediatric Use

The safety and effectiveness of MYQORZO in pediatric patients have not been established.

8.5 Geriatric Use

There were 57 (40%) MYQORZO-treated patients, aged 65 years and older, in SEQUOIA-HCM *[see Clinical Studies (14)]*. Of the total number of MYQORZO-treated patients in SEQUOIA-HCM, 45 (32%) patients were 65 to 74 years of age, and 12 (8%) patients were 75 years of age and older.

No overall differences in safety or effectiveness of MYQORZO have been observed between patients 65 years of age and older and younger adult patients.

10 OVERDOSAGE

Clinical Experience and Effects

There have been no reports of overdose with MYQORZO. Cardiovascular effects of overdose may include reduced LVEF, heart failure, and hypotension. Signs and symptoms of heart failure such as dyspnea, fatigue, leg edema, or elevations in NT-proBNP should prompt an evaluation of cardiac function *[see Warnings and Precautions (5.1)]*.

Management

Discontinue MYQORZO treatment. Provide medically supportive measures to maintain hemodynamic stability and monitor left ventricular function.

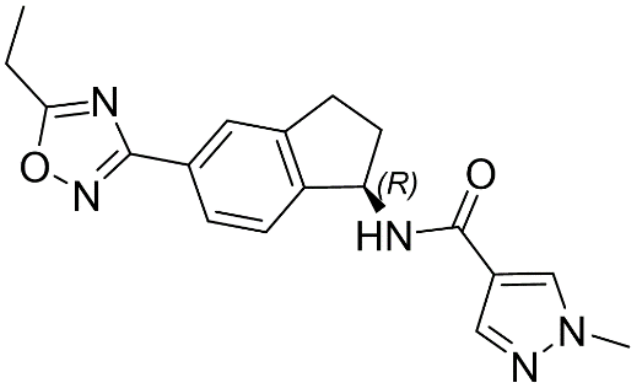
Consider contacting the Poison Help line (1-800-222-1222) or a medical toxicologist for additional overdose management recommendations.

11 DESCRIPTION

MYQORZO tablets for oral use contain aficamten, a cardiac myosin inhibitor.

The chemical name of aficamten is (R)-N-(5-(5-ethyl-1,2,4-oxadiazol-3-yl)-2,3-dihydro-1*H*-inden-1-yl)-1-methyl-1*H*-pyrazole-4-carboxamide. The molecular formula is $C_{18}H_{19}N_5O_2$ and the molecular weight is 337.38 g/mol.

The structural formula of aficamten is:



Aficamten is a white to off-white to yellow to grey solid that is practically insoluble in water and aqueous buffers (pH 2 – 9), sparingly soluble in acetone and ethanol, and soluble in N-methylpyrrolidone (NMP).

MYQORZO is supplied as tablets containing 5 mg, 10 mg, 15 mg, or 20 mg of aficamten per tablet as the active ingredient and the following inactive ingredients: croscarmellose sodium, hydroxypropyl cellulose, magnesium stearate (non-bovine), mannitol, microcrystalline cellulose, and sodium lauryl sulfate. The tablet coating contains carmine, FD&C Blue No. 2, glyceryl mono and dicaprylate, polyvinyl alcohol, polyvinyl alcohol graft polyethylene copolymer, talc and titanium dioxide.

12 CLINICAL PHARMACOLOGY

12.1 Mechanism of Action

Aficamten is an allosteric and reversible inhibitor of cardiac myosin motor activity. Aficamten reduces the force generated by myosin at the cardiac sarcomere, which contributes to the pathophysiology of HCM. In patients with HCM, myosin inhibition with aficamten reduces cardiac contractility and left ventricular outflow tract (LVOT) obstruction.

12.2 Pharmacodynamics

Left Ventricular Ejection Fraction and Left Ventricular Outflow Tract Obstruction

In SEQUOIA-HCM, reductions in LVOT-G were observed at 2 weeks after initiating treatment, and LVOT-G continued to decrease through Week 8. The reduction in LVOT-G was sustained through the remainder of the 24-week trial. At Week 24, the mean (SD) change from baseline in resting and Valsalva LVOT-G was -35 (29) mmHg and -48 (37) mmHg, respectively, for the MYQORZO group and 4 (35) mmHg and 2 (35) mmHg, respectively, for the placebo group. The reduction in LVOT-G was accompanied by a greater decrease in LVEF in the MYQORZO group compared to the placebo group with mean (SD) change from baseline in LVEF of -7 (7)% and -2 (6)%, respectively. Four weeks after discontinuation of treatment, LVEF and Valsalva LVOT-G returned to baseline.

Cardiac Structure

In SEQUOIA-HCM, maximal left ventricular wall thickness, left ventricular mass index (LVMI) and left atrial volume index (LAVI) were measured at baseline and Week 24. At Week 24, the mean (SD) maximal left ventricular wall thickness decreased in the MYQORZO group by -0.2 (0.3) cm and in the placebo group by -0.1 (0.3) cm. LVMI decreased in the MYQORZO group by -5.3 (24) g/m^2 and increased in the placebo group by 5.8 (25) g/m^2. At Week 24, LAVI decreased in the MYQORZO group by -2.6 (8) mL/m^2 and increased in the placebo group by 1.4 (8) mL/m^2. The clinical significance of these findings is unknown.

Cardiac Biomarkers
In SEQUOIA-HCM, reductions in NT-proBNP were observed in the MYQORZO group starting at Week 2 and were sustained through Week 24. At Week 24, NT-proBNP was 80% lower with MYQORZO compared to placebo (proportion of geometric mean ratio between MYQORZO and placebo, 0.20 [95% CI: 0.17, 0.23]). At both Week 12 and 24, troponin I was 40% lower with MYQORZO compared to placebo (proportion of geometric mean ratio between the two groups, 0.57 [95% CI: 0.51, 0.64]).

The clinical significance of the NT-proBNP and troponin findings is unknown.

Cardiac Electrophysiology
The results of a thorough QT study demonstrated a lack of QTc prolongation across the therapeutic concentration range of MYQORZO. At a single dose of 50 mg (similar exposure to 20 mg daily dosing to steady-state), the upper limit of the predicted placebo-corrected change from baseline in QT interval corrected by Fridericia (ΔΔQTcF) 90% confidence interval for MYQORZO was <10 msec.

12.3 Pharmacokinetics

Aficamten exposure increases dose proportionally following single doses of 1 mg to 75 mg and multiple once daily doses of 5 mg to 20 mg. Aficamten pharmacokinetics was comparable between healthy subjects and patients with oHCM. Geometric mean accumulation ratios for aficamten were similar across dose levels and ranged from 4.6 to 4.8. Steady state is predicted to be reached following approximately 17 days of once daily dosing of MYQORZO in patients with oHCM.

Absorption
Aficamten is rapidly absorbed with a median time to maximum concentration (T_{max}) of 1.5 to 2.0 hours. The bioavailability of aficamten after oral administration is unknown.

Effect of Food
No clinically significant differences in aficamten AUC and C_{max} were observed following its administration with a high-fat, high-calorie meal.

Distribution
Aficamten is approximately 90% bound to plasma proteins and demonstrates a volume of distribution of 313 L. The blood to plasma ratio of aficamten was 0.94.

Elimination
Aficamten has a median terminal half-life (t½) of approximately 80 hours in patients with oHCM. At steady-state, the peak-to-trough plasma concentration ratio with once daily dosing is approximately 1.2. The total clearance is 2.6 L/hr and the renal clearance is <0.1% of total clearance.

Metabolism
Aficamten is extensively metabolized in humans, primarily through CYP2C9 with contributions by CYP3A, CYP2D6, and CYP2C19. Aficamten is primarily metabolized to two pharmacologically inactive metabolites, CK-3834282 and CK-3834283, that circulate at approximately 56% and 103% of parent in plasma, respectively.

Excretion
Following a single 20 mg radiolabeled dose of aficamten, 32% (0.06% unchanged aficamten) was excreted in urine and 58% (5.1% unchanged aficamten) was excreted in feces.

Specific Populations
No clinically meaningful differences in the pharmacokinetics of aficamten were observed based on age (18 – 83 years), race, mild to moderate hepatic impairment (Child-Pugh Classes A and B), or mild to moderate renal impairment (estimated glomerular filtration rate (eGFR) ≥30 mL/min

normalized by body surface area and based on the modification of diet in renal disease (MDRD) equation). The effects of severe renal (eGFR <30 mL/min based on MDRD equation) or hepatic (Child-Pugh Class C) impairment are unknown.

Small differences in aficamten exposure with sex and body weight were observed. Female patients demonstrated a 31% higher mean exposure (AUC) compared to male patients. The highest body weight quartile of patients (median: 105 kg) demonstrated a 34% lower aficamten mean exposure (AUC) than the lowest body weight quartile of patients (median: 64 kg). These differences are not clinically significant.

Drug Interaction Studies

Clinical Studies and Model-Informed Approaches

Inhibitors and Inducers of CYP enzymes: The predicted and observed effect of coadministration of select CYP inhibitors and inducers on the plasma exposure of aficamten is provided in Table 3.

Table 3: In Vivo Observed and Predicted Effects of CYP Inhibitors and Inducers on Aficamten Plasma Exposure

Concomitant Medication	Dosage	Effect on Aficamten Mean AUC
Inhibitors of Individual CYP Enzymes		
Sulfaphenazole[1] - *Strong CYP2C9 Inhibitor*	2000 mg once daily	AUC: ↑ 113%
Itraconazole[2] - *Strong CYP3A Inhibitor*	200 mg once daily	AUC: ↑ 26%
Paroxetine[2] – *Strong CYP2D6 Inhibitor*	40 mg once daily	AUC: ↑ 27%
Inhibitors of Multiple CYP Enzymes		
Fluconazole[2] - *Strong CYP2C19, Moderate CYP3A, and Moderate CYP2C9 Inhibitor*	400 mg once daily	AUC: ↑ 278%
	150 mg x 2 doses; separated by 48 hours	AUC: No change
Fluvoxamine[1] - *Strong CYP2C19, Weak-to-Moderate CYP3A, Weak CYP2C9, and Weak CYP2D6 Inhibitor*	150 mg twice daily	AUC: ↑ 116%
Voriconazole[1] - *Strong CYP3A, Moderate CYP2C19, and Weak CYP2C9 inhibitor*	200 mg twice daily	AUC: ↑ 94%
Fluoxetine[2] - *Strong CYP2D6 and Strong CYP2C19 Inhibitor*	40 mg once daily	AUC: ↑ 32%
CYP Enzyme Inducers		
Rifampin[1] - *Strong CYP3A, Strong CYP2C19, and Moderate CYP2C9 Inducer*	600 mg once daily	AUC: ↓ 79%
Carbamazepine[2] – *Moderate-to-strong CYP3A and Weak CYP2C9 Inducer*	300 mg twice daily	AUC: ↓ 51%

1. Predicted via physiologically-based pharmacokinetic modeling.
2. Observed.

P-gp Transporter Substrates: Concomitant administration of a single 20 mg dose of aficamten increased dabigatran (administered as dabigatran etexilate) mean AUC_{inf} by 26%.

In Vitro Drug Interaction Studies

CYP enzymes: Aficamten is not expected to inhibit CYP1A2, CYP2B6, CYP2C8, CYP2C19, CYP2D6, or CYP3A4 at clinically relevant concentrations. Aficamten is not expected to induce CYP1A2, CYP2B6 or CYP3A4 at clinically relevant concentrations.

Transporter systems: Aficamten is not a sensitive substrate of drug transporters. Aficamten is not expected to inhibit BCRP, OCT2, OCT3, OAT1, OAT3, OATP1B1, OATP1B3, MATE1 or MATE2-K drug transporters at clinically relevant concentrations.

12.5 Pharmacogenomics

Aficamten is primarily metabolized by the polymorphic enzyme, CYP2C9, with lesser contributions by other polymorphic enzymes (e.g., CYP2D6 and CYP2C19) *[see Clinical Pharmacology (12.3)]*. The impact of CYP2C9 genetic variants on the pharmacokinetics of aficamten has not been directly evaluated, though, a similar effect on exposure is expected as seen with strong CYP2C9 inhibitors *[see Drug Interactions (7.1)]*.

No additional dosage modifications are required for patients who are CYP2C9 poor metabolizers, as MYQORZO dosage titration and modifications account for CYP2C9 metabolizer status *[see Dosage and Administration (2.2) and (2.3)]*.

13 NONCLINICAL TOXICOLOGY

13.1 Carcinogenesis, Mutagenesis, Impairment of Fertility

Carcinogenesis

The carcinogenic potential of aficamten was assessed in a 26-week study in transgenic rasH2 mice and a 2-year study in Sprague Dawley rats. There was no evidence of carcinogenicity with oral administration of aficamten at dose levels up to 2.0 mg/kg/day in the mouse or 3 mg/kg/day in the rat (corresponding to 4-fold the MRHD based on free AUC).

Mutagenesis

Aficamten was not genotoxic in the in vitro bacterial (Ames) mutagenicity assay or the combined in vivo micronucleus assay and comet assay in rat.

Impairment of Fertility

In a fertility and early embryonic development toxicology study in the rat, aficamten was administered at doses up to 6 mg/kg/day in females and 3 mg/kg/day in males, with no drug-related effects on mating, fertility, estrous cycling, male reproductive assessments, or early embryonic survival.

14 CLINICAL STUDIES

The efficacy of MYQORZO was evaluated in SEQUOIA-HCM (NCT05186818), a phase 3, multicenter, randomized, double-blind, placebo-controlled study in 282 adults (142 aficamten, 140 placebo) with symptomatic New York Heart Association (NYHA) class II and III oHCM, LVEF ≥60%, and resting and post-Valsalva peak LVOT-G ≥30 and ≥50 mmHg at screening, respectively. Patients who completed SEQUOIA-HCM were eligible to participate in an ongoing, open-label, single-arm extension study (FOREST-HCM). Patients with a known infiltrative or storage disorder causing cardiac hypertrophy such as Noonan syndrome, Fabry disease or amyloidosis were excluded.

Patients were randomized in a 1:1 ratio to receive either MYQORZO or placebo once daily for 24 weeks. Randomization was stratified by use of beta-blockers (yes or no) and cardiopulmonary exercise testing (CPET) exercise modality (treadmill or bicycle)

At baseline, 76% of the randomized patients were NYHA class II and 24% were NYHA class III. The mean peak oxygen uptake (pVO_2) by CPET at baseline was 18.5 mL/kg/min with 55% using treadmill and 45% using bicycle, respectively. At baseline, the median LVEF was 76%, the mean resting LVOT-G was 55 mmHg, the mean Valsalva LVOTG was 83 mmHg, and mean Kansas City Cardiomyopathy Questionnaire – Clinical Summary Score (KCCQ-CSS) was 74. At baseline, 61% of patients were on beta-blockers, 29% were on non-dihydropyridine calcium channel blockers, 13% were on disopyramide, and 15% were not taking any background medication for oHCM.

Groups were balanced with respect to age (mean 59 years; range 18 to 84 years), sex (59% male), race (79% White, 19% Asian and 1% Black or African American), body mass index (mean 28 kg/m^2), heart rate (mean 66 bpm) and blood pressure (mean 124/74 mmHg).

Patients were initiated on MYQORZO at a dose of 5 mg once daily. Doses were individually titrated (or sham-titrated if on placebo) at Week 2, 4 and 6 if Valsalva LVOT-G was ≥30 mmHg and LVEF was ≥55% in 5 mg dose increments up to a maximum dose of 20 mg once daily. At Week 24, in the MYQORZO group, 46% of patients were on the 20 mg dose, 35% were on the 15 mg dose, 15% were on the 10 mg dose and 4% were on the 5 mg dose.

Primary Endpoint - Peak Oxygen Uptake (pVO_2)

In SEQUOIA-HCM, the primary endpoint of change from baseline in pVO_2 to Week 24 was greater with MYQORZO compared to placebo, as shown in Table 4.

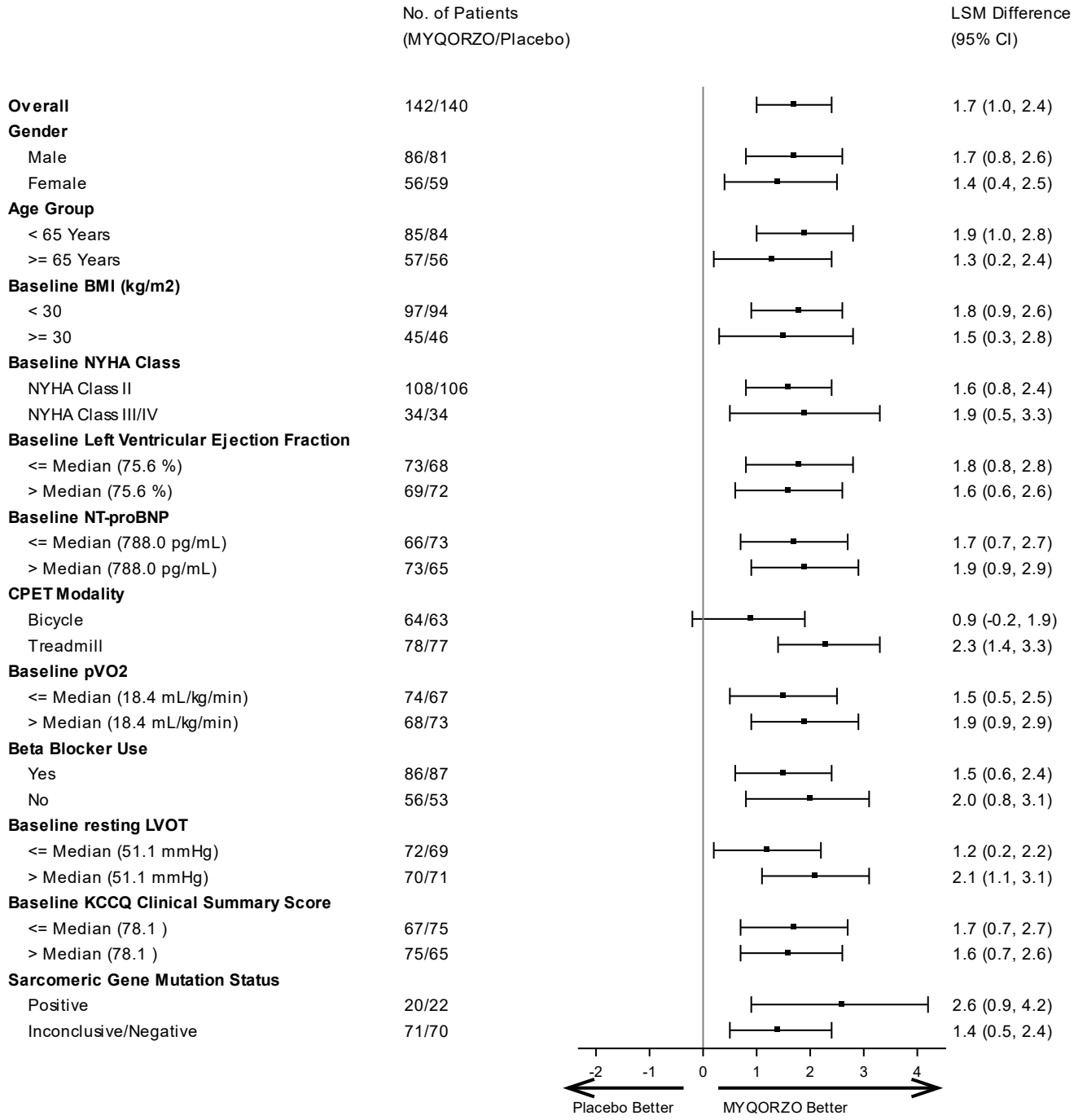
Table 4: Change from Baseline in pVO_2 by CPET at Week 24 in SEQUOIA-HCM

pVO_2	**MYQORZO (N = 142)**	**Placebo (N = 140)**
Baseline (mL/min/kg), mean (SD)	18.4 (4.4)	18.6 (4.5)
Change from baseline to Week 24		
LS mean (SE)[1]	1.7 (0.3)	0.0 (0.3)
LS mean difference vs. placebo (95% CI)[1]	1.7 (1.0, 2.4)	
p-value	<0.0001	

1. LS mean (SE), and LS mean difference (95% CI) were estimated from an ANCOVA model. A placebo-based multiple imputation approach was used to impute the missing data.

A range of demographic characteristics, baseline disease characteristics, and baseline concomitant medications (e.g., use of beta-blockers) were examined for their influence on outcomes. Results of the primary endpoint analysis consistently favored MYQORZO across all subgroups analyzed (Figure 1).

Figure 1: Forest Plot for Change in Baseline pVO_2 by CPET at Week 24 by Subgroups in SEQUOIA-HCM



BMI: Body Mass Index; NYHA: New York Heart Association; NT-proBNP: N-terminal pro-B-type natriuretic peptic; CPET: Cardiac Pulmonary Exercise Test; pVO2: Peak Oxygen Uptake; LVOT: left ventricular outflow tract; KCCQ CSS: Kansas City Cardiomyopathy Questionnaire – Clinical Summary Score.

Note: The figure above presents effects in various subgroups, all of which are baseline characteristics. The 95% confidence limits that are shown do not take into account the number of comparisons made and may not reflect the effect of a particular factor after adjustment for all other factors. Apparent homogeneity or heterogeneity among groups should not be over-interprete

Secondary Endpoints

The treatment effects of MYQORZO on health status, functional capacity, and LVOT obstruction were assessed by change in the KCCQ-CSS, proportion of patients with ≥1 class improvement in NYHA functional class, change from baseline in Valsalva LVOT-G, proportion of patients with Valsalva LVOT-G ≤30 mmHg, duration of eligibility for septal reduction therapy (SRT), and change in total workload during CPET. At Week 24, patients receiving MYQORZO had greater improvement compared to the placebo group across all secondary points (Table 5, Figure 2 and Figure 3).

Table 5. Results of Secondary Efficacy Endpoints in SEQUOIA-HCM

Endpoints	**MYQORZO N=142**	**Placebo N=140**	**LS Mean Difference (95% CI)**	**p-Value**
Mean Change from Baseline in KCCQ-CSS[1,3]				
Week 12	11.1 (0.9)	4.0 (0.9)	7.0 (4.6, 9.5)	<0.0001
Week 24	11.6 (1.0)	4.3 (1.0)	7.3 (4.6, 10.1)	<0.0001
Mean Change from Baseline in Valsalva LVOT-G (mmHg)[1]				
Week 12	-45.0 (2.4)	2.7 (2.4)	-47.7 (-54.5, -41.0)	<0.0001
Week 24	-46.7 (2.5)	2.2 (2.5)	-48.8 (-55.7, -42.0)	<0.0001
Mean Duration of SRT Eligibility[1]				
Days spent SRT-eligible during 24 Weeks of Treatment	35 (8)	113 (8)	−78 (−100, −56)	<0.0001
Mean Change from Baseline in Total Workload (Watts) during CPET[1]				
Week 24	13.1 (2.1)	0.9 (2.2)	12.2 (6.3, 18.1)	<0.0001
Endpoints	**MYQORZO N=142**	**Placebo N=140**	**Odds Ratio (95% CI)**	**p-Value**
Proportion of Patients with ≥1 NYHA Class Improvement[2]				
Week 12	69 (48.6)	25 (17.9)	OR: 4.6 (2.6, 8.4)	<0.0001
Week 24	83 (58.5)	34 (24.3)	OR: 4.4 (2.6, 7.6)	<0.0001
Proportion of Patients with Valsalva LVOT-G <30 (mmHg)[2]				
Week 12	73 (51.4)	8 (5.7)	OR: 16.9 (7.7, 37.2)	<0.0001
Week 24	70 (49.3)	5 (3.6)	OR: 22.9 (8.8, 59.6)	<0.0001

CPET: Cardiac Pulmonary Exercise Test; KCCQ CSS: Kansas City Cardiomyopathy Questionnaire – Clinical Summary Score; NYHA: New York Heart Association; LVOT-G: left ventricular outflow tract gradient; SRT: septal reduction therapy.

1. LS means (SE) and LS mean difference (95% CI) was estimated from an ANCOVA model (mixed model with repeated measures was used for longitudinal data) for continuous endpoints. Placebo-based multiple imputation approach was used to impute the missing data.
2. The number (percentage) of responders and common Odds Ratio (OR) obtained using Cochran-Mantel-Haenszel method (95% CI) are presented for binary endpoints.
3. The KCCQ-CSS score ranges from 0 to 100, with higher scores indicating better health status.

Figure 2: Change from Baseline in KCCQ-CSS through Week 24

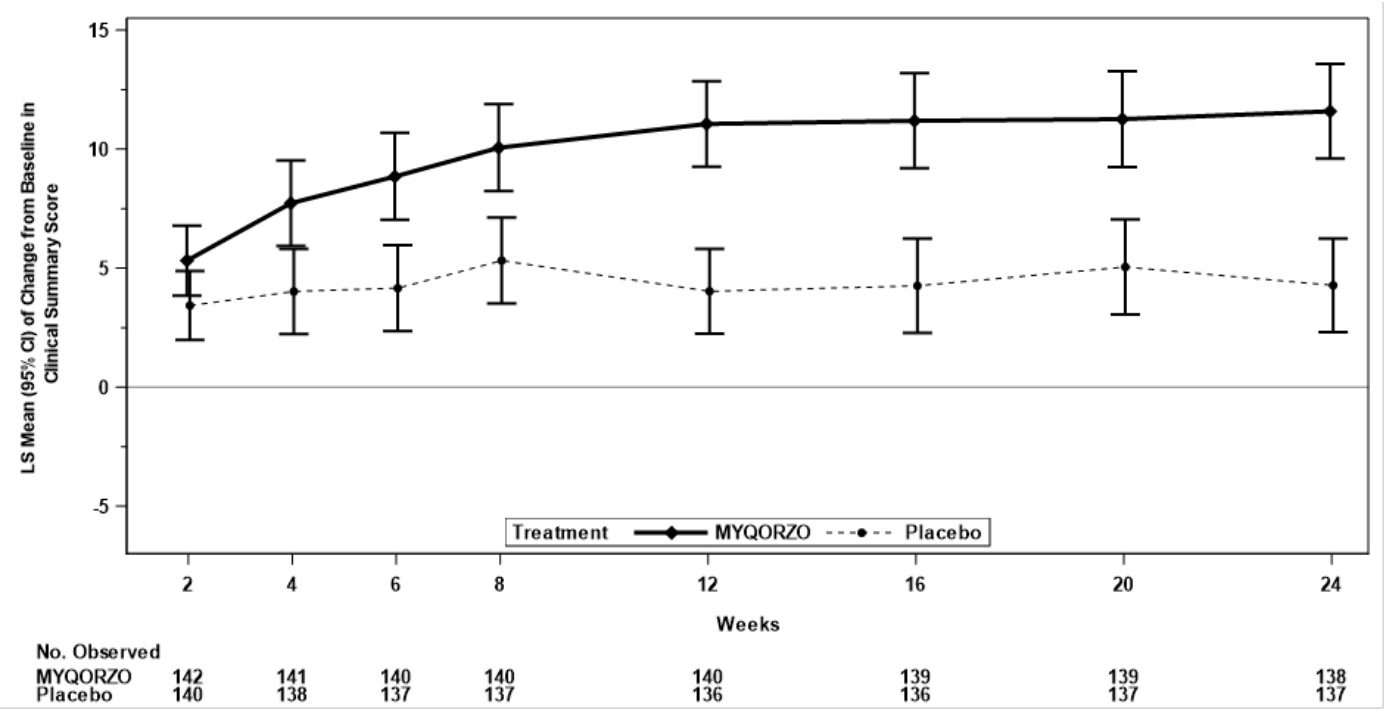
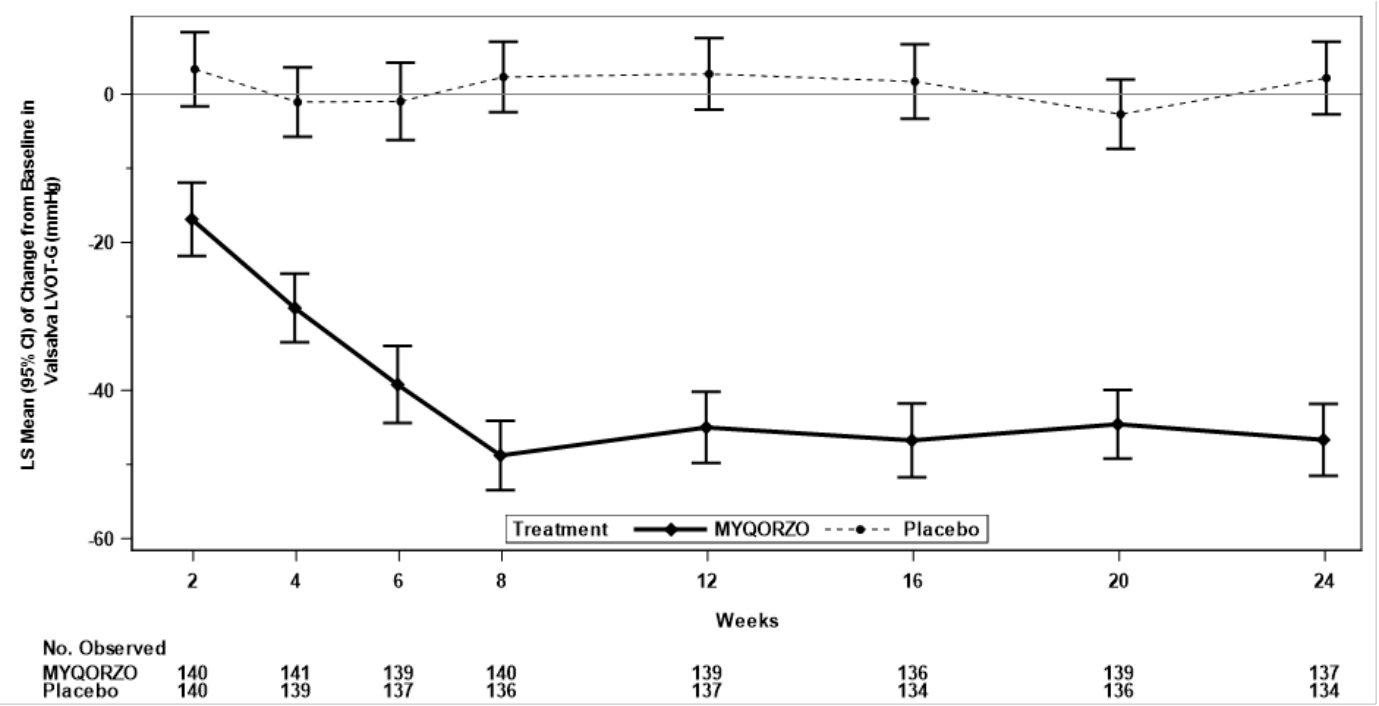


Figure 3: Change from Baseline in Valsalva LVOT-G through Week 24



16 HOW SUPPLIED/STORAGE AND HANDLING

MYQORZO is supplied as purple, film-coated tablets containing 5 mg, 10 mg, 15 mg, or 20 mg aficamten. The tablets are debossed on one side with “CK” and the other side with “5”, “10”, “15” and “20” for the 5, 10, 15, and 20 mg strength, respectively.

Tablets are supplied in bottles with child-resistant closure as follows:

5 mg round tablets	Bottles of 30 tablets	NDC: 82112-105-30
10 mg triangular tablets	Bottles of 30 tablets	NDC: 82112-110-30
15 mg pentagonal tablets	Bottles of 30 tablets	NDC: 82112-115-30
20 mg oval tablets	Bottles of 30 tablets	NDC: 82112-120-30

Storage

Store at 20°C to 25°C (68°F to 77°F); excursions permitted between 15°C to 30°C (59°F to 86°F) *[see USP controlled Room Temperature].*

17 PATIENT COUNSELING INFORMATION

Advise the patient and/or caregiver to read the FDA-approved patient labeling (Medication Guide).

Heart Failure

Inform patients that cardiac function monitoring using echocardiography must be performed before and while on treatment to monitor for signs and symptoms of heart failure *[see Dosage and Administration (2.1), Warnings and Precautions (5.1)].* Advise patients to report any signs or symptoms of heart failure immediately to their healthcare provider.

MYQORZO REMS Program

MYQORZO is available only through a restricted program called the MYQORZO REMS Program *[see Warnings and Precautions (5.2)]*. Inform the patient of the following notable requirements:

Patients must enroll in the program and comply with ongoing monitoring requirements *[see Warnings and Precautions (5.1) and (5.2)]*

MYQORZO is only prescribed by certified healthcare providers and only dispensed from certified pharmacies participating in the program. Provide patients with the telephone number and website for information on how to obtain the product *[see Warnings and Precautions (5.2)].*

Pregnancy

Advise patients that there is a pregnancy safety study for MYQORZO. Advise patients to report pregnancy to their prescriber if they become pregnant while receiving MYQORZO or within 3 weeks after their last dose of MYQORZO *[see Use in Specific Populations (8.1)]*.

Drug Interactions

Advise patients to inform their healthcare provider of all concomitant medications they take, prior to and during MYQORZO treatment.

Instructions for Taking MYQORZO

MYQORZO tablets should be swallowed whole. Advise patients that if they miss a dose of MYQORZO, it should be taken as soon as possible on the same day. The next scheduled dose should be taken at the usual time the following day. The patient should not take two doses on the same day.

Distributed By:
Cytokinetics Incorpo

South San Francisco, CA 94080

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Patent Information: https://cytokinetics.com/patents
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MEDICATION GUIDE
MYQORZO (my-kor-zo)
(aficamten)
tablets, for oral use

What is the most important information I should know about MYQORZO?

MYQORZO can cause serious side effects, including:

- **Heart failure,** a condition where the heart cannot pump with enough force. Heart failure is a serious condition that can lead to death. You must have echocardiograms before and during treatment with MYQORZO and monitor for signs and symptoms of heart failure. People who develop a serious illness such as a serious infection or who develop a new or worsening irregular heartbeat have a greater risk of heart failure during treatment with MYQORZO.

  Tell your healthcare provider or get medical help right away if you develop new or worsening:
  - shortness of breath
  - swelling in your legs
  - a racing sensation in your heart (palpitations)
  - fatigue
  - chest pain
  - rapid weight gain
- **The risk of heart failure is also increased when MYQORZO is taken with certain other medicines.** Tell your healthcare provider about the medicines you take, both prescribed and obtained over-the-counter, before and during your treatment with MYQORZO.
- **Because of the risk of heart failure, MYQORZO is only available through a restricted distribution program called the MYQORZO Risk Evaluation and Mitigation Strategy (REMS) Program.**
  - Your healthcare provider must be enrolled in the MYQORZO REMS Program for you to be prescribed MYQORZO.
  - Before you start treatment with MYQORZO, you must enroll in the MYQORZO REMS Program. Talk to your healthcare provider about how to enroll in the program. You will be given information about the program when you enroll.
  - Before you take MYQORZO, your healthcare provider and pharmacist will make sure you understand how to take MYQORZO safely, which will include returning for echocardiograms when advised by your healthcare provider. MYQORZO can only be dispensed by a certified pharmacy that participates in the MYQORZO REMS Program. Your healthcare provider can give you information on how to find a certified pharmacy.
  - If you have any questions about the MYQORZO REMS Program, ask your healthcare provider, go to www.MYQORZOREMS.com, or call 1-844-285-7367.
- See **"What are the possible side effects of MYQORZO?"** for information about side effects.

What is MYQORZO?

MYQORZO is a prescription medicine used to treat adults with symptomatic obstructive cardiomyopathy (oHCM) to improve functional capacity and symptoms.

It is not known if MYQORZO is safe and effective in children.

Who should not take MYQORZO?

Do not take MYQORZO if you take a medicine called rifampin.

Before taking MYQORZO, tell your healthcare provider about all of your medical conditions, including if you:

- are pregnant or plan to become pregnant. It is not known if MYQORZO can cause harm to your unborn baby. Tell your healthcare provider if you become pregnant during treatment or within 3 weeks after the last dose of MYQORZO. There is a pregnancy safety study for MYQORZO. Your healthcare provider should report your pregnancy exposure to Cytokinetics, Inc.
- are breastfeeding or plan to breastfeed. It is not known if MYQORZO passes into your breast milk. Talk to your healthcare provider about the best way to feed your baby during treatment with MYQORZO.

Before and during MYQORZO treatment, tell your healthcare provider about all the medicines you take, including prescription and over-the-counter medicines, vitamins, and herbal supplements. Taking MYQORZO with certain medicines may lead to increased levels of MYQORZO in your blood and increase your risk of heart failure. **Do not stop or change the dose of a medicine or start a new medicine without telling your healthcare provider.**

Especially tell your healthcare provider if you take the medicines:

- fluconazole (if used for more than 3 days)
- voriconazole
- fluvoxamine

How should I take MYQORZO?

- Take MYQORZO exactly as your healthcare provider tells you to take it.
- Do not change your dose of MYQORZO without talking to your healthcare provider first.
- Take MYQORZO 1 time a day, at about the same time each day,
- Take MYQORZO with or without meals.
- Swallow the MYQORZO tablet whole.
- If you miss a dose of MYQORZO, take it as soon as possible and take your next dose at your regularly scheduled time the next day. Do not take 2 doses on the same day to make up for a missed dose.
- Your healthcare provider may change your dose or temporarily stop your treatment with MYQORZO if you have certain side effects.
- If you take too much MYQORZO, call your healthcare provider or Poison Control Help Line at 1-800-222-1222 or go to the nearest hospital emergency room right away.

What are the possible side effects of MYQORZO?

MYQORZO can cause serious side effects including:

- **Heart failure.** See **"What is the most important information I should know about MYQORZO?**

The most common side effect of MYQORZO is high blood pressure (hypertension).

Call your doctor for medical advice about side effects. You may report side effects to FDA at 1-800-FDA-1088.
You may also report side effects to Cytokinetics at 1-833-633-2986.

How should I store MYQORZO?

- Store MYQORZO at room temperature between 68°F to 77°F (20°C to 25°C).
- MYQORZO comes in a bottle with a child-resistant closure.

Keep MYQORZO and all medicines out of the reach of children.

General information about the safe and effective use of MYQORZO.

Medicines are sometimes prescribed for purposes other than those listed in a Medication Guide. Do not use MYQORZO for a condition for which it was not prescribed. Do not give MYQORZO to other people, even if they have the same symptoms that you have; it may harm them. You can ask your pharmacist or healthcare provider for information about MYQORZO that is written for health professionals.

What are the ingredients in MYQORZO?

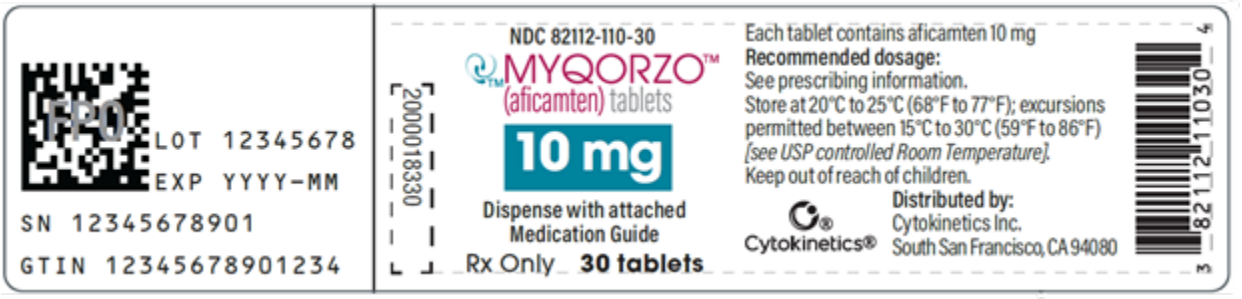
Active ingredients: aficamten
Inactive ingredients: croscarmellose sodium, hydroxypropyl cellulose, magnesium stearate (non-bovine), mannitol, microcrystalline cellulose, and sodium lauryl sulfate.
The tablet coating contains: carmine, FD&C Blue No. 2, glyceryl mono and dicaprylate, polyvinyl alcohol, polyvinyl alcohol graft polyethylene copolymer, talc and titanium dioxide.

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Patent information: https://cytokinetics.com/patents

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This Medication Guide has been approved by the U.S. Food and Drug Administration. Issued: 12/2025


LOT 12345678
EXP YYYY-MM
SN 12345678901
GTIN 12345678901234
NDC 82112-105-30
MYQORZO™
(aficamten) tablets
5 mg
Dispense with attached Medication Guide
Rx Only 30 tablets
Each tablet contains aficamten 5 mg
Recommended dosage:
See prescribing information.
Store at 20°C to 25°C (68°F to 77°F); excursions permitted between 15°C to 30°C (59°F to 86°F) [see USP controlled Room Temperature].
Keep out of reach of children.
Distributed by:
Cytokinetics Inc.
South San Francisco, CA 94080
Cytokinetics®


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EXP YYYY-MM
SN 12345678901
GTIN 12345678901234
NDC 82112-110-30
MYQORZO™
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10 mg
Dispense with attached Medication Guide
Rx Only 30 tablets
Each tablet contains aficamten 10 mg
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Distributed by:
Cytokinetics Inc.
South San Francisco, CA 94080
Cytokinetics®


LOT 12345678
EXP YYYY-MM
SN 12345678901
GTIN 12345678901234
NDC 82112-115-30
MYQORZO™
(aficamten) tablets
15 mg
Dispense with attached Medication Guide
Rx Only 30 tablets
Each tablet contains aficamten 15 mg
Recommended dosage:
See prescribing information.
Store at 20°C to 25°C (68°F to 77°F); excursions permitted between 15°C to 30°C (59°F to 86°F) [see USP controlled Room Temperature].
Keep out of reach of children.
Distributed by:
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LOT 12345678
EXP YYYY-MM
SN 12345678901
GTIN 12345678901234
NDC 82112-120-30
MYQORZO™
(aficamten) tablets
20 mg
Dispense with attached Medication Guide
Rx Only 30 tablets
Each tablet contains aficamten 20 mg
Recommended dosage:
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Keep out of reach of children.
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Risk Evaluation and Mitigation Strategy (REMS) Document
MYQORZO™ (aficamten) REMS

I. Administrative Information

Risk: heart failure due to systolic dysfunction
Application Number: NDA 219083
Application Holder: Cytokinetics, Inc.
Initial REMS Approval: 12/2025

II. REMS Goal

The goal of the MYQORZO REMS is to mitigate the risk of heart failure due to systolic dysfunction.

Objective 1: Healthcare providers monitor left ventricular ejection fraction (LVEF) by echocardiogram during treatment according to the frequency described in the Prescribing Information to detect heart failure due to systolic dysfunction.

III. REMS Requirements

Cytokinetics, Inc. must ensure that healthcare providers, patients, pharmacies, and wholesalers-distributors comply with the following requirements:

1. Healthcare providers who prescribe MYQORZO must:	
To become certified to prescribe	1. Review the drug's Prescribing Information. 2. Review the REMS Overview and Education Program for Healthcare Providers and Pharmacies. 3. Successfully complete the Healthcare Provider Knowledge Assessment and submit it to the REMS. 4. Enroll by completing and submitting the Healthcare Provider Enrollment Form to the REMS.
Before treatment initiation (first dose)	5. Counsel the patient on the risk of heart failure due to systolic dysfunction using the Patient Guide. Provide a copy of the material to the patient. 6. Assess the patient's cardiovascular status and the appropriateness of initiating treatment by obtaining an echocardiogram. Document and submit confirmation of review of the patient's echocardiogram results and authorization for treatment to the REMS using the Patient Enrollment Form. 7. Enroll the patient by completing and submitting the Patient Enrollment Form to the REMS.

During treatment, between 2 to 8 weeks after treatment initiation and each dose adjustment, then every 3 or 6 months thereafter (depending on echocardiogram results)	8. Counsel the patient on the risk of heart failure due to systolic dysfunction using the Patient Guide. 9. Assess the patient's cardiovascular status and the appropriateness of continuing treatment by obtaining an echocardiogram. 10. Document and submit confirmation of review of the patient's echocardiogram results and authorization for treatment to the REMS using the Patient Monitoring Form.
At all times	11. Report adverse events of heart failure due to systolic dysfunction to Cytokinetics, Inc.
2. Patients who are prescribed MYQORZO must:	
Before treatment initiation (first dose)	1. Receive counseling from the healthcare provider on the risk of heart failure due to systolic dysfunction (where the heart cannot pump with enough force) using the Patient Guide. 2. Get an echocardiogram to check their heart. 3. Enroll in the REMS by completing the Patient Enrollment Form with the healthcare provider. Enrollment information will be provided to the REMS.
During treatment, between 2 to 8 weeks after treatment initiation and each dose adjustment, then every 3 or 6 months thereafter (depending on echocardiogram results)	4. Receive counseling from the healthcare provider on the risk of heart failure due to systolic dysfunction (where the heart cannot pump with enough force) using the Patient Guide. 5. Get an echocardiogram to check their heart.
At all times	6. Inform the healthcare provider or seek other medical attention if there are new or worsening symptoms of heart failure.
3. Pharmacies that dispense MYQORZO must:	
To become certified to dispense	1. Designate an Authorized Representative to carry out the certification process and oversee implementation and compliance with the REMS on behalf of the pharmacy. 2. Have the Authorized Representative review the Prescribing Information, REMS Overview, and Education Program for Healthcare Providers and Pharmacies. 3. Have the Authorized Representative enroll by completing and submitting the Pharmacy Enrollment Form to the REMS. 4. Train all relevant staff involved in dispensing MYQORZO using the REMS Overview and Education Program for Healthcare Providers and Pharmacies.
Before dispensing	5. Obtain authorization to dispense each prescription from the MYQORZO REMS to verify that the healthcare provider is certified, the patient is enrolled, the healthcare provider

	has authorized the patient to receive MYQORZO, and the requested dose is an allowable dose. 6. For patients initiating treatment, undergoing a dose adjustment, or re-initiating dose titration: Dispense no more than a 30 days' supply. 7. For patients on a maintenance dose: Dispense no more than a 90 days' supply.
At all times	8. Report adverse events of heart failure due to systolic dysfunction to Cytokinetics, Inc. 9. Not distribute, transfer, loan, or sell MYQORZO, except to a certified pharmacy. 10. Maintain records of dispensing information and provide such data during audits conducted by Cytokinetics, Inc. or a third party acting on behalf of Cytokinetics, Inc. 11. Maintain records of completion of the REMS training by relevant staff. 12. Maintain records that all processes and procedures are in place and are being followed. 13. Comply with audits conducted by Cytokinetics, Inc. or a third party acting on behalf of Cytokinetics, Inc. to ensure that all processes and procedures are in place and are being followed.
To maintain certification to dispense	14. Any new Authorized Representative must enroll by completing and submitting the Pharmacy Enrollment Form to the REMS.
4. Wholesalers-distributors that distribute MYQORZO must:	
To be able to distribute	1. Establish processes and procedures to ensure that the drug is distributed only to certified pharmacies. 2. Train all relevant staff involved in distributing MYQORZO on the REMS requirements.
At all times	3. Distribute only to certified pharmacies. 4. Maintain records of drug distribution for all MYQORZO shipments. 5. Maintain records that all processes and procedures are in place and are being followed. 6. Comply with audits conducted by Cytokinetics, Inc. or a third party acting on behalf of Cytokinetics, Inc. to ensure that all processes and procedures are in place and are being followed.

Cytokinetics, Inc. must provide training to healthcare providers who prescribe MYQORZO.
The training includes the following educational materials: REMS Overview, Education Program for Healthcare Providers and Pharmacies, and Healthcare Provider Knowledge Assessment. The training must be available online and by hard copy via fax and mail.

Cytokinetics, Inc. must provide training to pharmacies that dispense MYQORZO.
The training includes the following educational materials: REMS Overview and Education Program for Healthcare Providers and Pharmacies. The training must be available online and by hard copy via fax and mail.

To support REMS operations, Cytokinetics, Inc. must:

1. Authorize dispensing for each patient based on verifying the healthcare provider is certified, the pharmacy is certified, and the patient is enrolled and authorized to receive MYQORZO on the following schedule:
    - Treatment Initiation
        i. If the Patient Enrollment Form is not completed, the patient is not authorized to receive drug until the completed form is received.
        ii. The initial dispense authorization is valid for 3 months from the date the completed Patient Enrollment Form is received.
        iii. Additional dispenses at the starting dose are authorized within the first 60 days, up to a total of a 60 days' supply.
    - Dose Titration / Dose Adjustments
        i. After treatment initiation or a dose adjustment, the Patient Monitoring Form indicating the patient is authorized to continue receiving MYQORZO must be received within 60 days.
        ii. If a valid Patient Monitoring Form is not received, the patient is not authorized to receive the drug until the completed Patient Monitoring Form is received.
    - Maintenance Dose
        i. The Patient Monitoring Form indicating the patient is authorized to continue MYQORZO must be received every 6 months (for LVEF ≥55%) or every 3 months (for LVEF <55% and ≥50%) from the most recent Patient Monitoring Form.
        ii. If a valid Patient Monitoring Form is not received, the patient is not authorized to receive the drug until the completed Patient Monitoring Form is received.

2. Establish and maintain a REMS website, www.MYQORZOREMS.com. The REMS website must include the capability to complete healthcare provider and pharmacy certification online; the capability to enroll and manage patients online, including completion of the Patient Monitoring Form; the capability to review patient enrollment and authorization status and healthcare provider certification status, as well as obtain authorization to dispense; and the option to print the Prescribing Information, Medication Guide, and REMS materials. All product websites for consumers and healthcare providers must include prominent REMS-specific links to the REMS website. The REMS website must not link back to the promotional product website(s).

3. Make the REMS website fully operational and all REMS materials available through www.MYQORZOREMS.com and the MYQORZO REMS Call Center at the time that MYQORZO first becomes commercially available.

4. Establish and maintain a REMS Call Center for REMS participants at 1-844-285-7367.

5. Establish and maintain a validated, secure database of all REMS participants who are enrolled and/or certified in the MYQORZO REMS.

6. Ensure healthcare providers and pharmacies are able to complete the certification process online and by fax.

7. Ensure healthcare providers are able to complete the Patient Enrollment Form online and by fax.

8. Ensure healthcare providers are able to complete the Patient Monitoring Form online and by fax.

9. Ensure pharmacies are able to obtain authorization to dispense online and by phone.

10. Provide the REMS Overview, Education Program for Healthcare Providers and Pharmacies, Healthcare Provider Enrollment Form or Pharmacy Enrollment Form, and Prescribing Information to REMS participants who (1) attempt to prescribe or dispense MYQORZO and are not yet certified or (2) inquire about how to become certified.

11. Notify healthcare providers and pharmacies within two (2) business days when they become certified in the REMS.

12. Notify healthcare providers within two (2) business days when patient enrollment is complete.

13. Provide certified healthcare providers access to the database of enrolled patients and certified pharmacies.

14. Provide certified pharmacies access to the database of certified healthcare providers and enrolled patients.

15. Provide authorized wholesalers-distributors access to the database of certified pharmacies.

To ensure REMS participants' compliance with the REMS, Cytokinetics, Inc. must:

16. Verify annually that each Authorized Representative's name and contact information corresponds to those of a current designated Authorized Representative for the certified pharmacy. If different and there is no other certified Authorized Representative, the pharmacy must be required to certify a new Authorized Representative.

17. Maintain adequate records to demonstrate that the REMS requirements have been met, including but not limited to records of the following: MYQORZO distribution and dispensing, certification of healthcare providers and pharmacies, enrollment of patients, completed Patient Monitoring Forms, and audits of certified pharmacies and wholesalers-distributors. These records must be readily available for FDA inspections.

18. Establish and maintain a plan for addressing noncompliance with the REMS requirements.

19. Monitor healthcare providers, pharmacies, and wholesalers-distributors on an ongoing basis to ensure the REMS requirements are being met. Take corrective action if noncompliance is identified, including de-certification.

20. Audit all pharmacies no later than 180 calendar days after they become certified, and annually thereafter, to ensure all REMS processes and procedures are in place, functioning, and support the REMS requirements.

21. Audit all wholesalers-distributors no later than 180 calendar days after they place their first order of MYQORZO, and annually thereafter, to ensure all REMS processes and procedures are in place, functioning, and support the REMS requirements.

22. Take reasonable steps to improve operations of and compliance with the requirements in the MYQORZO REMS based on monitoring and evaluation of the MYQORZO REMS.

IV. REMS Assessment Timetable

Cytokinetics, Inc. must submit REMS assessments at 12 months and annually thereafter from the date of the initial approval of the REMS (12/19/2025). To facilitate the inclusion of as much information as possible while allowing reasonable time to prepare the submission, the reporting interval covered by each assessment should conclude no earlier than 60 calendar days before the submission date for that assessment. Cytokinetics, Inc. must submit each assessment so that it will be received by the FDA on or before the due date.

V. REMS Materials

The following materials are part of the MYQORZO REMS:

Enrollment Forms:

Healthcare Providers:

1. Healthcare Provider Enrollment Form

Patient:

2. Patient Enrollment Form

Pharmacy:

3. Pharmacy Enrollment Form

Training and Educational Materials:

Healthcare Providers:

4. Education Program for Healthcare Providers and Pharmacies
5. REMS Overview
6. Healthcare Provider Knowledge Assessment

Patient:

7. Patient Guide

Pharmacy:

8. Education Program for Healthcare Providers and Pharmacies
9. REMS Overview

Patient Care Form:

10. Patient Monitoring Form

Other Material:

11. REMS Website

VI. Statutory Elements

This REMS is approved under section 505-1 of the Federal Food, Drug, and Cosmetic Act (FD&C Act) (21 U.S.C. 355-1) and consists of the following elements:

1. Elements to Assure Safe Use (ETASU):
    - Healthcare providers who prescribe MYQORZO are specially certified under 505-1(f)(3)(A)
    - Pharmacies that dispense MYQORZO are specially certified under 505-1(f)(3)(B)
    - MYQORZO is dispensed to patients with evidence or other documentation of safe-use conditions under 505-1(f)(3)(D)
    - Each patient using MYQORZO is subject to certain monitoring under 505-1(f)(3)(E)
2. Implementation System
3. Timetable for Submission of Assessments

MYQORZO™ REMS

Healthcare Provider Enrollment Form

MYQORZO™ (aficamten) 5·10·15·20 mg tablets

Instructions:

1. Review the Prescribing Information, **REMS Overview**, and the **Education Program for Healthcare Providers and Pharmacies**.
2. Successfully complete the **Healthcare Provider Knowledge Assessment** by receiving a passing score of 100% and submit to the REMS.
3. Enroll by completing and submitting the **Healthcare Provider Enrollment Form** to the REMS:
   - Online at **www.MYQORZOREMS.com**, or
   - By fax to 1-844-285-7399

Complete all required fields on this form to avoid a delay in the enrollment process. Upon completion of these steps, the REMS will notify you of your successful certification within 2 business days.

Healthcare Provider Information (Fields marked * are REQUIRED)

*First Name:	Middle Initial:	*Last Name:

*Credentials: ☐ MD ☐ DO ☐ PA ☐ NP ☐ Other (please specify):

*Specialty: ☐ Cardiology ☐ Internal/General Medicine ☐ Other (please specify):

*Healthcare Provider NPI #:

*Practice/Facility Name:

*Address:

*City:	*State:	*ZIP Code:

*Phone Number: Area Code/Telephone Number	*Fax Number: Area Code/Fax Number

*Email:

*Preferred Method of Contact (please specify): ☐ Email ☐ Phone ☐ Fax	Preferred Time of Contact: ☐ AM ☐ PM

REMS Support Staff Information (Optional): Healthcare providers may grant administrative rights to up to four REMS support staff per affiliated healthcare setting. REMS support staff may fill out **Patient Enrollment Forms** and/or **Patient Monitoring Forms** on behalf of the certified healthcare provider online or by hard copy. The certified healthcare provider of record is responsible for reviewing, signing, and submitting these forms to the REMS.

I, the healthcare provider, grant administrative rights to the REMS support staff listed below, affiliated with the healthcare setting named above, and understand that I must review all paperwork prior to submitting to the REMS.

First Name:	Last Name:
Phone Number:	Email:
First Name:	Last Name:
Phone Number:	Email:
First Name:	Last Name:
Phone Number:	Email:
First Name:	Last Name:
Phone Number:	Email:

Reference ID: 5715575

MYQORZO™ REMS

Healthcare Provider Enrollment Form

MYQORZO™ (aficamten) 5·10·15·20 mg tablets

Healthcare Provider Agreement

By completing, signing, and submitting this form, I agree and acknowledge that:

To become certified, I must:

- Review the drug's Prescribing Information, **REMS Overview**, and the **Education Program for Healthcare Providers and Pharmacies**.
- Successfully complete the **Healthcare Provider Knowledge Assessment** and submit it to the REMS.
- Enroll by completing and submitting the **Healthcare Provider Enrollment Form** to the REMS.

Before treatment initiation (first dose), I must:

- Counsel the patient on the risk of heart failure due to systolic dysfunction using the **Patient Guide**. Provide a copy of the material to the patient.
- Assess the patient's cardiovascular status and the appropriateness of initiating treatment by obtaining an echocardiogram. Document and submit confirmation of an echocardiogram and authorization for treatment to the REMS using the **Patient Enrollment Form**.
- Enroll the patient by completing and submitting the **Patient Enrollment Form** to the REMS.

During treatment: 2 to 8 weeks after treatment initiation and each dose change, then every 3 or 6 months thereafter (depending on echocardiogram results), I must:

- Counsel the patient on the risk of heart failure due to systolic dysfunction using the **Patient Guide**.
- Assess the patient's cardiovascular status and the appropriateness of continuing treatment by obtaining an echocardiogram.
- Document and submit confirmation of review of the patient's echocardiogram results and authorization for treatment to the REMS using the **Patient Monitoring Form**.

At all times, I must:

- Report adverse events of heart failure due to systolic dysfunction to Cytokinetics, Inc.

Provide Signature Below

By signing below, I acknowledge the above agreements and my obligations as a MYQORZO healthcare provider to comply with MYQORZO REMS requirements, and I understand my personally identifiable information provided above will be shared with Cytokinetics, Inc., its agents, contractors, and affiliates and entered into a healthcare provider database for the MYQORZO REMS. I agree that I may be contacted in the future by mail, email, fax, and/or phone concerning MYQORZO, the MYQORZO REMS, and other MYQORZO programs and services.

*Healthcare Provider Signature:	*Date (MM/DD/YYYY):

Note: MYQORZO REMS certified healthcare providers may add additional practices/facilities and or REMS support staff either through the MYQORZO REMS Website or by contacting the MYQORZO REMS Call Center.

If you have any questions about the MYQORZO REMS, please call the MYQORZO REMS Call Center at 1-844-285-7367. For MYQORZO REMS Call Center hours, visit www.MYQORZOREMS.com.

PHONE: 1-844-285-7367 FAX: 1-844-285-7399 www.MYQORZOREMS.com

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Reference ID: 5715575

MYQORZO™ REMS

Patient Enrollment Form

MYQORZO™ (aficamten) 5·10·15·20 mg tablets

Instructions:

Enroll in the REMS by completing the **Patient Enrollment Form** with your healthcare provider.

- Complete this form online at **www.MYQORZOREMS.com**, or
- Print and complete this form and submit by fax to 1-844-285-7399

Patient Information (Fields marked * are REQUIRED)

*First Name:	Middle Initial:	*Last Name:
*Date of Birth: Month/Day/Year		
*Address:		
*City:	*State:	*ZIP Code:
*Phone: Area Code/Telephone Number	Alternate Phone: Area Code/Telephone Number	
*Email:		
Preferred Method of Contact (please select one): ☐ Phone ☐ Email		
Person to Contact if Patient Is Not Available/ Secondary Contact:	Phone Number for Secondary Contact:	

Patient Agreement

Before treatment initiation (first dose), I must:

- Receive counseling from the healthcare provider on the risk of heart failure due to systolic dysfunction (where the heart cannot pump with enough force) using the **Patient Guide**.
- Get an echocardiogram to check my heart.
- Enroll in the REMS by completing the **Patient Enrollment Form** with my healthcare provider. Enrollment information will be provided to the REMS.

During treatment: 2 to 8 weeks after treatment initiation and each dose change, then every 3 or 6 months thereafter (depending on echocardiogram results), I must:

- Receive counseling from my healthcare provider on the risk of heart failure due to systolic dysfunction (where the heart cannot pump with enough force), using the **Patient Guide**.
- Get an echocardiogram to check my heart.

At all times, I will:

- Inform my healthcare provider or seek other medical attention if there are new or worsening symptoms of heart failure.

Provide Signature Below

By signing below, I acknowledge the above agreements and my obligations as a MYQORZO patient to comply with REMS requirements, and I understand that my protected health information will be stored in a secure and confidential database of all patients who receive MYQORZO in the United States. Cytokinetics, Inc. and its agents will use and share my protected health information for the MYQORZO REMS program and for FDA reporting. I agree that Cytokinetics, Inc. and its agents may contact me by phone, mail, or email to manage the MYQORZO REMS.

***Patient/Legal Guardian Signature:**	***Date (MM/DD/YYYY):**
Printed Legal Guardian Name (REQUIRED if not signed by the patient):	

Reference ID: 5715575

MYQORZO™ REMS

Patient Enrollment Form

MYQORZO™ (aficamten) 5·10·15·20 mg tablets

Certified Healthcare Provider Information

*First Name:	*Last Name:
*Healthcare Provider NPI #:	*Phone Number: Area Code/Telephone Number

Certified Healthcare Provider Acknowledgment

- I have counseled the patient on the risk of heart failure due to systolic dysfunction using the **Patient Guide**. A copy of the **Patient Guide** has been provided to the patient.
- I have assessed the patient's cardiovascular status and the appropriateness of initiating treatment by obtaining an echocardiogram.
- I understand that treatment must be initiated within three months of submitting this form. If treatment is not initiated within this timeframe, a new **Patient Enrollment Form** must be submitted.

Treatment Authorization (select one):*

☐ I authorize this patient to initiate or continue treatment with MYQORZO at **5 mg**, or

☐ I authorize this patient to continue treatment with MYQORZO at a different dose.

If authorizing a different dose, answer the following:

Authorized dose: ☐ 10 mg ☐ 15 mg ☐ 20 mg

Reason: ☐ Clinical trial patient on a stable dose

☐ Established international patient now treated in USA

***Certified Healthcare Provider Signature:**	***Date (MM/DD/YYYY):**

Note: MYQORZO REMS support staff may fill out this form on behalf of the certified healthcare provider of record. The certified healthcare provider of record is responsible for reviewing and submitting this form, as well as ensuring compliance with the REMS requirements. This includes overseeing the monitoring, evaluation, and management of each patient under their care.

If you have any questions about the MYQORZO REMS, please call the MYQORZO REMS Call Center at 1-844-285-7367. For MYQORZO REMS Call Center hours, visit www.MYQORZOREMS.com.

PHONE: 1-844-285-7367 FAX: 1-844-285-7399 www.MYQORZOREMS.com

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MYQORZO™ REMS

Pharmacy Enrollment Form

MYQORZO™ (aficamten) 5·10·15·20 mg tablets

Instructions:

1. Designate at least one, and up to two, Authorized Representatives to complete the certification process and oversee implementation and compliance with the REMS on behalf of the pharmacy.
2. Have each Authorized Representative:
   - Review the Prescribing Information, **REMS Overview**, and the **Education Program for Healthcare Providers and Pharmacies**.
   - Successfully complete and submit the **Pharmacy Enrollment Form** to the REMS:
     - Online at **www.MYQORZOREMS.com**, or
     - By fax to 1-844-285-7399

Complete all required fields on this form to avoid a delay in the enrollment process. Upon completion of these steps, the REMS will notify pharmacies of successful certification within 2 business days.

Pharmacy Information (Fields marked * are REQUIRED)

*Pharmacy Name:		
*Pharmacy NPI #:	*Pharmacy DEA #:	
*Pharmacy Address:		
*City:	*State:	*ZIP Code:
*Phone Number: Area Code/Telephone Number	*Fax Number: Area Code/Fax Number	

Authorized Representative Information

*First Name:	Middle Initial:	*Last Name:
*Title/Position:		
*Phone Number: Area Code/Telephone Number	*Fax Number: Area Code/Fax Number	
*Email:		
*Preferred Method of Contact: (please select one): ☐ Email ☐ Phone ☐ Fax		

Secondary Pharmacy Contact

First Name:	Middle Initial:	Last Name:
Title/Position:		
Phone Number: Area Code/Telephone Number	Fax Number: Area Code/Fax Number	
Email:		

Reference ID: 5715575

MYQORZO™ REMS

Pharmacy Enrollment Form

MYQORZO™ (aficamten) 5·10·15·20 mg tablets

Pharmacy Agreement

By completing, signing, and submitting this form, I agree and acknowledge that:

As the Pharmacy Authorized Representative, I must:

- Review the Prescribing Information, **REMS Overview**, and **Education Program for Healthcare Providers and Pharmacies**.
- Enroll in the REMS by completing and submitting the **Pharmacy Enrollment Form** to the REMS.
- Train all relevant staff involved in dispensing MYQORZO using the **REMS Overview**, and **Education Program for Healthcare Providers and Pharmacies**.

Before dispensing, all pharmacy staff must:

- Obtain authorization to dispense each prescription from the MYQORZO REMS to verify that the healthcare provider is certified, the patient is enrolled, the healthcare provider has authorized the patient to receive MYQORZO, and the requested dose is an allowable dose.
- Dispense no more than a 30 days' supply for patient initiating treatment, undergoing a dose adjustment, or re-initiating dose titration.
- Dispense no more than a 90 days' supply for patients on a maintenance dose.

At all times, my pharmacy must:

- Report adverse events of heart failure due to systolic dysfunction to Cytokinetics, Inc.
- Not distribute, transfer, loan, or sell MYQORZO, except to a certified pharmacy.
- Maintain records of dispensing information and provide such data during audits conducted by Cytokinetics, Inc. or a third party acting on behalf of Cytokinetics, Inc.
- Maintain records of completion of the REMS training by relevant staff.
- Maintain records that all processes and procedures are in place and are being followed.
- Comply with audits conducted by Cytokinetics, Inc. or a third party acting on behalf of Cytokinetics, Inc. to ensure that all processes and procedures are in place and are being followed.

To maintain certification to dispense, any new Authorized Representative must:

- Enroll by completing and submitting the **Pharmacy Enrollment Form** to the REMS.

Provide Signature Below

By signing below, I acknowledge the above agreements and my obligations as a MYQORZO pharmacy to comply with MYQORZO REMS requirements, and I understand my personally identifiable information provided above will be shared with Cytokinetics, Inc., its agents, contractors, and affiliates and entered into a pharmacy database for the MYQORZO REMS. I agree that I may be contacted in the future by mail, email, fax, and/or phone concerning MYQORZO, the MYQORZO REMS, and other MYQORZO programs and services.

***Authorized Representative Signature:**	***Date (MM/DD/YYYY):**

If you have any questions about the MYQORZO REMS, please call the MYQORZO REMS Call Center at 1-844-285-7367. For MYQORZO REMS Call Center hours, visit www.MYQORZOREMS.com.

PHONE: 1-844-285-7367 FAX: 1-844-285-7399 www.MYQORZOREMS.com

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MYQORZO™ REMS

Education Program for Healthcare Providers and Pharmacies

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Reference ID: 5715575

Cytokinetics

Welcome to the MYQORZO REMS **Education Program for Healthcare Providers and Pharmacies**

To prescribe MYQORZO (aficamten), healthcare providers must become certified in the MYQORZO Risk Evaluation and Mitigation Strategy (REMS), which includes reviewing this **Education Program for Healthcare Providers and Pharmacies**.

- Only certified healthcare providers are eligible to prescribe MYQORZO.
- MYQORZO can only be dispensed by certified pharmacies.
- Patients must be enrolled in the MYQORZO REMS to receive MYQORZO.

Reference ID: 5715575

MYQORZO Indication and Mechanism of Action

Indication:

MYQORZO is a cardiac myosin inhibitor indicated for the treatment of adults with symptomatic obstructive hypertrophic cardiomyopathy (oHCM).

Mechanism of Action:

Aficamten is an allosteric and reversible inhibitor of cardiac myosin motor activity. Aficamten reduces the force generated by myosin at the cardiac sarcomere, which contributes to the pathophysiology of HCM. In HCM patients, myosin inhibition with aficamten reduces cardiac contractility and left ventricular outflow tract (LVOT) obstruction.

See full Prescribing Information.

Healthcare providers must report adverse events of heart failure due to systolic dysfunction associated with MYQORZO to Cytokinetics at 1-833-633-2986.

Reference ID: 5715575

MYQORZO REMS
Risk Information

MYQORZO Boxed Warning

WARNING: RISK OF HEART FAILURE

MYQORZO reduces left ventricular ejection fraction (LVEF) and can cause heart failure due to systolic dysfunction.

Echocardiogram assessments are required prior to and during treatment with MYQORZO to monitor for systolic dysfunction. Initiation of MYQORZO in patients with LVEF <55% is not recommended. Decrease the dose of MYQORZO if LVEF is <50% and ≥40%. Interrupt the dose of MYQORZO if LVEF <40% or if the patient experiences heart failure symptoms or worsening clinical status due to systolic dysfunction.

Because of the risk heart failure due to systolic dysfunction, MYQORZO is available only through a restricted program under a Risk Evaluation and Mitigation Strategy (REMS) called the MYQORZO REMS Program.

Risk of Heart Failure Due to Systolic Dysfunction

Assess the patient's cardiovascular status and appropriateness of initiating or continuing treatment and adjust the MYQORZO dose accordingly

- Patients who experience a serious intercurrent illness (e.g., serious infection) or arrhythmia (e.g., new or uncontrolled atrial fibrillation) may be at greater risk of developing systolic dysfunction and heart failure.
- Asymptomatic LVEF reduction, intercurrent illnesses, and arrhythmias require additional monitoring considerations.
- New or worsening arrhythmia, dyspnea, chest pain, fatigue, leg edema, or elevations in N-terminal pro-B-type natriuretic peptide (NT-proBNP) may be signs and symptoms of heart failure and should prompt an evaluation of cardiac function.
- Initiation of MYQORZO in patients with LVEF <55% is not recommended.

Reference ID: 5715575

Recommended Dosage and Administration

Initiation or up-titration of MYQORZO in patients with LVEF <55% is not recommended.

- The recommended starting dose of MYQORZO is 5 mg orally once daily
- Increase the dose every 2 to 8 weeks by 5 mg increments until a maintenance dose or the maximum recommended dose of 20 mg once daily is achieved
    - The maintenance dose of MYQORZO is individualized based on the patient's LVEF and left ventricular outflow tract (LVOT) gradient
- Recommendations for dosing based on LVEF and LVOT gradient criteria are in Table 1 (right)

Table 1: Dose Adjustment of MYQORZO

LVEF	Valsalva LVOT Gradient	Dose Adjustment
≥55%	≥30 mm Hg	Increase dose by 5 mg (up to the maximum dose of 20 mg once daily)
≥55%	<30 mm Hg	Maintain Dose
<55% and ≥50%	Any	Maintain Dose
<50% and ≥40%	Any	Decrease dose by 5 mg* Interrupt treatment for 7 days for 5 mg dose
<40%	Any	Interrupt treatment for at least 7 days

*Decrease as follows: 20 mg to 15 mg; 15 mg to 10 mg; 10 mg to 5 mg.

Reference ID: 5715575

Recommended Dosage and Administration

The Use of Echocardiography in MYQORZO Dosing and Administration

Regular LVEF and Valsalva LVOT gradient assessment is required for titration to achieve an appropriate target Valsalva LVOT gradient, while maintaining LVEF ≥50%.

- An echocardiographic assessment should be **performed between 2 to 8 weeks**
  - after initiation of treatment,
  - after any dose adjustment, or
  - after treatment interruption
- After a treatment interruption, resume treatment at the starting dose of 5 mg when LVEF ≥55% and re-initiate dose titration
- After the maintenance dose has been established:
  - assess LVEF and Valsalva LVOT gradient every 6 months, or
  - every 3 months in patients with LVEF <55% to ≥50%
- Consider monitoring LVEF and adjust dose per Table 1 of the Prescribing Information, as needed, in patients during intercurrent illness (e.g., severe infection), new arrhythmia (e.g., new or uncontrolled atrial fibrillation or other uncontrolled tachyarrhythmia), or any other conditions that may impair systolic function
  - Do not increase dose until intercurrent illness or new arrhythmia has resolved or stabilized

Reference ID: 5715575

MYQORZO REMS Overview

MYQORZO REMS Overview

A REMS is a drug safety program that the U.S. Food and Drug Administration (FDA) can require for certain medicines to ensure that the benefits of a drug outweigh its risks.

Because of the risk of heart failure due to systolic dysfunction, MYQORZO is only available through a restricted program called the MYQORZO REMS.

The objective of the MYQORZO REMS is:

- Healthcare providers monitor left ventricular ejection fraction (LVEF) by echocardiogram during treatment according to the frequency described in the Prescribing Information to detect heart failure due to systolic dysfunction.

Reference ID: 5715575

Healthcare Provider Requirements

Healthcare Provider Requirements

MYQORZO can only be prescribed by certified healthcare providers.

How Does a Healthcare Provider Become Certified in the MYQORZO REMS?

To become certified in the MYQORZO REMS, healthcare providers must:

1. **Review** the MYQORZO Prescribing Information, **REMS Overview**, and **Education Program for Healthcare Providers and Pharmacies** (this presentation)
2. **Successfully complete** the **Healthcare Provider Knowledge Assessment** and submit it to the REMS.
3. **Enroll** by completing and submitting the **Healthcare Provider Enrollment Form** to the REMS.

   Complete certification:
   - Online at www.MYQORZOREMS.com, or
   - By faxing the completed forms to 1-844-285-7399

Upon completion of these steps, healthcare providers will be notified within 2 business days when they are certified in the MYQORZO REMS.

The healthcare provider certification confirmation will also include instructions for the healthcare provider to gain access to the REMS portal where they can enter patient monitoring information and add REMS support staff.

Reference ID: 5715575

Healthcare Provider Requirements

What Are the Healthcare Provider Requirements of the MYQORZO REMS?

To prescribe MYQORZO, certified healthcare providers must:

Before treatment initiation (first dose):

- Counsel the patient on the risk of heart failure due to systolic dysfunction using the **Patient Guide** and provide a copy to the patient.
- Assess the patient's cardiovascular status and the appropriateness of initiating treatment based on an echocardiogram.
  - Document and submit confirmation of an echocardiogram and authorization for treatment to the REMS using the **Patient Enrollment Form**.
  - Enroll the patient by completing and submitting the **Patient Enrollment Form** to the REMS.

During treatment, between 2 to 8 weeks after treatment initiation and each dose change, then every 3 or 6 months thereafter (depending on echocardiogram results):

- Counsel the patient on the risk of heart failure due to systolic dysfunction using the **Patient Guide**.
- Assess the patient's cardiovascular status and the appropriateness of continuing treatment by obtaining an echocardiogram.
- Document and submit confirmation of review of the patient's echocardiogram results and authorization for treatment to the REMS using the **Patient Monitoring Form**.

At all times:

- Report adverse events of heart failure due to systolic dysfunction to Cytokinetics, Inc.

Reference ID: 5715575

Healthcare Provider Requirements

Patient Enrollment Form Submission

The **Patient Enrollment Form** is used to document and confirm the following:

- Patient has been counselled on the risk of heart failure due to systolic dysfunction
- Healthcare provider has reviewed the patient's echocardiogram results and has authorized treatment including the starting dose

Treatment Initiation & Dispense Limits

Treatment with MYQORZO **must** begin within **3 months** of submitting the **Patient Enrollment Form**.

- Note: If treatment is not initiated within 3 months, the patient will be moved to a deactivated status, and a new **Patient Enrollment Form** must be submitted to proceed.

Single dispense limits are **capped** at a **30-day supply** during initiation and titration phases.

- Note: The REMS system will deny any dispense request that exceeds the applicable supply limit.

Reference ID: 5715575

Healthcare Provider Requirements

Patient Monitoring Form Completion

The **Patient Monitoring Form** is used to document and confirm the following:

- Certified healthcare provider has reviewed the patient's echocardiogram results in accordance with the Prescribing Information
- The patient's most recent LVEF
- If the patient has experienced a clinical heart failure event requiring clinical intervention or hospitalization since the last submitted form
- Action that will be taken to the patient's dose based on the clinical visit
- If the patient is authorized to continue treatment

Patient Monitoring Form Submission Timing

Treatment Initiation

- Within 60 days of initiating the first dose

Titration/Dose Changes

- Within 60 days following any dose change

Maintenance Dose

- For patients with LVEF ≥55% every 6 months from the most recent **Patient Monitoring Form**
- For patients with LVEF <55% and ≥50% every 3 months from the most recent **Patient Monitoring Form**

Maintenance Phase Dispense Limits

Single dispense limits are **capped** at a **90-day supply**.

Important: Submit the **Patient Monitoring Form** as soon as possible after completing the echocardiogram. Failure to submit the **Patient Monitoring Form** according to the required schedule may result in dispensing holds and potentially treatment interruptions.

Reference ID: 5715575

Healthcare Provider Requirements

Patient Monitoring Form Completion for Patients With Documented LVEF <40%

Treatment should be interrupted for at least 7 days per the Prescribing Information.

The REMS enforces the following additional **Patient Monitoring Form** submission requirement and dispensing limitations:

- **Recovery Confirmation:** Before treatment can be reinitiated, a new **Patient Monitoring Form** documenting LVEF ≥55% must be submitted to the REMS
- **Dispensing Limitation:** No REMS Dispense Authorization (RDA) will be issued until recovery to LVEF ≥55% is confirmed via **Patient Monitoring Form**
- **Restart Protocol:** Treatment must be reinitiated at the starting dose of 5 mg once daily, with an echocardiogram required within 2 to 8 weeks

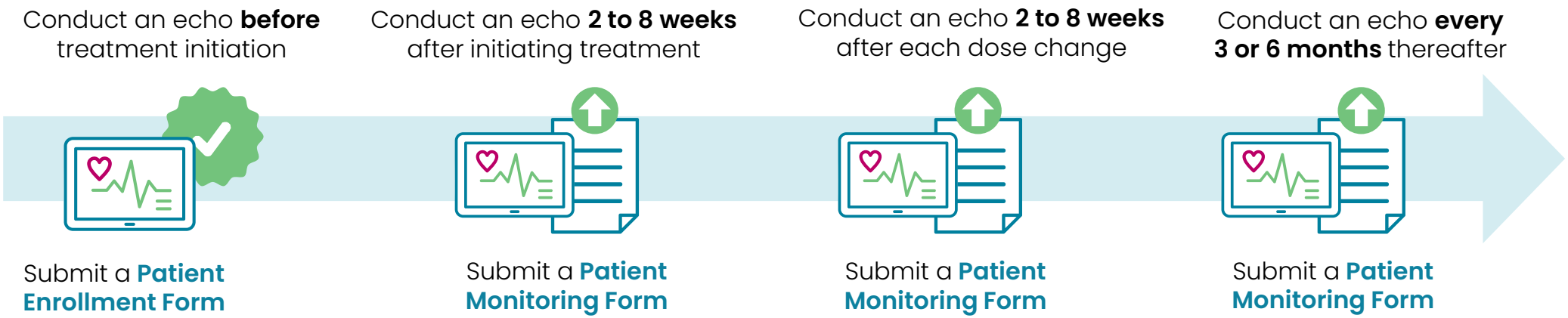
Reference ID: 5715575

Healthcare Provider Requirements

Echocardiogram & Patient Form Submission Timing Illustration

When prescribing MYQORZO, echocardiograms are required prior to initiation of treatment, after treatment initiation, after a dose change, and for routine monitoring. As depicted in the illustration below,

- An echocardiogram and **Patient Enrollment Form** are required prior to the patient initiating treatment.
- An echocardiogram is required 2 to 8 weeks after treatment initiation and any dose change, then every 3 or 6 months thereafter. A **Patient Monitoring Form** is required after each echocardiogram to confirm the patient's cardiovascular status has been assessed and the appropriateness of continuing treatment.



Reference ID: 5715575

Healthcare Provider Requirements

REMS Support Staff

To allow for flexibility in documentation management common within the healthcare delivery system and normal medical practice, the REMS allows for certified healthcare providers to designate up to four (4) REMS support staff per affiliated healthcare setting to complete certain REMS required documentation.

The certified healthcare provider of record is responsible for compliance with the REMS requirements, including monitoring, evaluation, and management of each patient under their care.

- REMS support staff can be added by the certified healthcare provider:
  - When completing and submitting the **Healthcare Provider Enrollment Form**
  - On the online portal accessible from www.MYQORZOREMS.com
  - By calling the REMS Call Center at 1-844-285-7367
- REMS support staff can be removed by the certified healthcare provider:
  - On the online portal accessible from www.MYQORZOREMS.com
  - By calling the REMS Call Center at 1-844-285-7367

Reference ID: 5715575

Healthcare Provider Requirements

REMS Healthcare Provider Delegate

To allow for flexibility in patient management common within the healthcare delivery system and normal medical practice, the REMS allows for certified healthcare providers to designate other certified healthcare providers as healthcare provider delegates to complete REMS-required documentation on their behalf.

Delegates must be enrolled and certified in REMS and attest to the requirements of the REMS.

- REMS healthcare provider delegates can be added or removed by the certified healthcare provider:
  - On the online portal accessible from www.MYQORZOREMS.com
  - By calling the REMS Call Center at 1-844-285-7367

Reference ID: 5715575

Healthcare Provider Requirements

REMS Responsibilities

Once appointed by a certified healthcare provider, REMS support staff and delegates can complete certain activities on behalf of the certified healthcare provider of record online or by hard copy as shown in the table below.

REMS Responsibilities	Certified Healthcare Provider	Healthcare Provider Delegate	Support Staff
Become REMS-certified	Yes	Yes	No
Counsel the patient on the risk of heart failure due to systolic dysfunction using the **Patient Brochure**	Yes	Yes	No
Assess the patient's cardiovascular status and the appropriateness of initiating or continuing treatment by obtaining an echocardiogram	Yes	Yes	No
Enter patient data on a **Patient Enrollment Form** and **Patient Monitoring Form** up to, but not including, signature	Yes	Yes	Yes
Sign the **Patient Enrollment Form** and **Patient Monitoring Form**	Yes	Yes	No

The certified healthcare provider of record or their delegate is responsible for reviewing, signing, and submitting REMS forms, as well as ensuring compliance with the REMS requirements; this includes overseeing the monitoring, evaluation, and management of each patient under their care.

Reference ID: 5715575

Healthcare Provider Requirements

Managing Healthcare Provider Changes for REMS-Enrolled Patients

When enrolled patients need to transition to a new healthcare provider, whether they are no longer under their original provider's care or are planning a switch, the transition can be facilitated through the REMS website or the REMS Call Center. The new healthcare provider must be certified in the REMS program before patient care can be transferred, ensuring continuity of care and compliance with REMS requirements.

- Patient reassignments can be initiated by current or new certified healthcare providers:
  - On the online portal accessible from www.MYQORZOREMS.com
  - By calling the REMS Call Center at 1-844-285-7367

Upon successful reassignment, the REMS will send a notification to both healthcare providers, confirming that the patient is now associated with the new healthcare provider.

Reference ID: 5715575

Pharmacy Requirements

MYQORZO will be dispensed by a limited number of specialty pharmacies.
All pharmacies that dispense MYQORZO must be certified in the REMS.

Pharmacy Requirements

How Does a Pharmacy Become Certified in the MYQORZO REMS?

To become certified to dispense, pharmacies must:

1. Designate an Authorized Representative to carry out the certification process and oversee implementation and compliance with the REMS on behalf of the pharmacy.
2. Have the Authorized Representative review the Prescribing Information, **REMS Overview**, and **Education Program for Healthcare Providers and Pharmacies** (this document).
3. Have the Authorized Representative enroll by completing and submitting the **Pharmacy Enrollment Form** to the REMS.
4. Train all relevant staff involved in dispensing MYQORZO using the **REMS Overview** and **Education Program for Healthcare Providers and Pharmacies**.

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Pharmacy Requirements

Eligible specialty pharmacies can complete certification:

- Online at www.MYQORZOREMS.com, or
- By faxing the completed form to 1-844-285-7399

Each pharmacy can designate up to 2 Authorized Representatives.

Pharmacies will be notified within two business days if they are certified to dispense MYQORZO.

The pharmacy certification confirmation will also include instructions for the Authorized Representative to gain access to the REMS portal where they can verify REMS requirements, obtain authorization to dispense MYQORZO, and add pharmacy staff.

Each certified Authorized Representative will have the ability to add staff on the online portal accessible from www.MYQORZOREMS.com, granting them access to the REMS portal. Any certified pharmacy staff member responsible for dispensing MYQORZO will receive unique login credentials.

Reference ID: 5715575

Pharmacy Requirements

What Are the Requirements of the MYQORZO REMS for Certified Pharmacies?

Before dispensing, all pharmacy staff must:

- Obtain authorization to dispense each prescription from the MYQORZO REMS online or by phone to verify that:
  - The healthcare provider is certified,
  - The patient is enrolled,
  - The healthcare provider has authorized the patient to receive MYQORZO, and
  - The requested dose is an allowable dose.
- Dispense no more than a 30 days' supply for patients initiating treatment, undergoing a dose adjustment, or re-initiating dose titration.
- Dispense no more than a 90 days' supply for patients on a maintenance dose.
- Each certified pharmacy will have the ability to obtain a REMS Dispense Authorization (RDA) for each prescription by:
  - Accessing a secure REMS verification portal through the REMS website, or
  - Contacting the REMS Call Center via phone at 1-844-285-7367

Reference ID: 5715575

Pharmacy Requirements

At all times, the pharmacy must:

- Report adverse events of heart failure due to systolic dysfunction to Cytokinetics, Inc.
- Not distribute, transfer, loan, or sell MYQORZO, except to a certified pharmacy.
- Maintain records of dispensing information and provide such data during audits conducted by Cytokinetics, Inc. or a third party acting on behalf of Cytokinetics, Inc.
- Maintain records of completion of the REMS training by relevant staff.
- Maintain records that all processes and procedures are in place and are being followed.
- Comply with audits carried out by Cytokinetics, Inc. or a third party acting on behalf of Cytokinetics, Inc. to ensure that all processes and procedures are in place and are being followed.

To maintain certification to dispense, any new Authorized Representative must:

- Enroll by completing and submitting the **Pharmacy Enrollment Form** to the REMS.

If the pharmacy opts to have a second Authorized Representative enroll, that Authorized Representative must complete and submit the **Pharmacy Enrollment Form** to the REMS.

Reference ID: 5715575

This concludes the MYQORZO REMS Education Program for Healthcare Providers and Pharmacies.

For more information or to obtain any REMS materials, visit www.MYQORZOREMS.com or call 1-844-285-7367. For MYQORZO REMS Call Center hours, visit www.MYQORZOREMS.com.

Report all adverse events, including those of heart failure due to systolic dysfunction, that occur in patients receiving MYQORZO to Cytokinetics, Inc. at 1-833-633-2986.

Phone: 1-844-285-7367 Fax: 1-844-285-7399 www.MYQORZOREMS.com

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MYQORZO™ REMS

REMS Overview

This overview describes the requirements of the MYQORZO REMS and the responsibilities of healthcare providers, pharmacists, and patients.

WHAT IS THE MYQORZO REMS?

A **R**isk **E**valuation and **M**itigation **S**trategy (**REMS**) is a strategy to manage known or potential risks associated with a drug product and is required by the Food and Drug Administration (FDA) to ensure the benefits of a drug outweigh its risks.

Because of the risk of heart failure due to systolic dysfunction, MYQORZO is available only through a restricted program called the MYQORZO REMS.

MYQORZO may increase the risk of heart failure due to systolic dysfunction; patients may experience heart failure at any time during treatment with MYQORZO.

What are the requirements of the MYQORZO REMS?

In order for patients to receive MYQORZO, healthcare providers, pharmacies, and patients must comply with the requirements of the MYQORZO REMS.

Healthcare Provider Requirements

Only healthcare providers certified in the MYQORZO REMS can prescribe MYQORZO.

Healthcare providers must:

Become Certified (One Time)	**To become certified to prescribe:** 1. Review the drug's Prescribing Information, **REMS Overview** (this document), and the **Education Program for Healthcare Providers and Pharmacies**. 2. Successfully complete the **Healthcare Provider Knowledge Assessment** and submit it to the REMS 3. Enroll by completing and submitting the **Healthcare Provider Enrollment Form** to the REMS. Upon completion of these steps, the REMS will notify the healthcare provider of successful certification within 2 business days.
Enroll Patients	**Before treatment initiation (first dose):** 5. Counsel the patient on the risk of heart failure due to systolic dysfunction using the **Patient Guide**. Provide a copy of the material to the patient. 6. Assess the patient's cardiovascular status and the appropriateness of initiating treatment by obtaining an echocardiogram. Document and submit confirmation of an echocardiogram and authorization for treatment to the REMS using the **Patient Enrollment Form**. 7. Enroll the patient by completing and submitting the **Patient Enrollment Form** to the REMS.

Monitor Patients	**During treatment, between 2 to 8 weeks after treatment initiation and each dose change, then every 3 or 6 months thereafter (depending on echocardiogram results):** 8. Counsel the patient on the risk of heart failure due to systolic dysfunction using the **Patient Guide**. 9. Assess the patient's cardiovascular status and the appropriateness of continuing treatment by obtaining an echocardiogram. 10. Document and submit confirmation of review of the patient's echocardiogram results and authorization for treatment to the REMS using the **Patient Monitoring Form**. The MYQORZO REMS will send reminders to the certified healthcare provider to submit the **Patient Monitoring Form**. Completed forms should be submitted to the MYQORZO REMS online at www.MYQORZOREMS.com or via fax to 1-844-285-7399. Failure to submit the **Patient Monitoring Form** at the required intervals could result in a delay in dispensing of MYQORZO to the patient.
At All Times	Report adverse events of heart failure due to systolic dysfunction to Cytokinetics, Inc.

Pharmacy Requirements

Only pharmacies certified in the MYQORZO REMS can dispense MYQORZO.

Pharmacies must:

Become Certified	**To become certified:** 1. Designate an authorized representative to carry out the certification process and oversee implementation and compliance with the REMS on behalf of the pharmacy. 2. Have the authorized representative review the Prescribing Information, **REMS Overview** (this document), and **Education Program for Healthcare Providers and Pharmacies**. 3. Have the authorized representative enroll by completing and submitting the **Pharmacy Enrollment Form** to the REMS. 4. Train all relevant staff involved in dispensing MYQORZO using the **REMS Overview** and **Education Program for Healthcare Providers and Pharmacies**.
Prior to Dispense	**Before dispensing, all pharmacy staff must:** 5. Obtain authorization to dispense each prescription from the MYQORZO REMS to verify that the healthcare provider is certified, the patient is enrolled, the healthcare provider has authorized the patient to receive the drug, and the requested dose is an allowable dose. 6. Dispense no more than a 30 days' supply for patient initiating treatment, undergoing a dose adjustment, or re-initiating dose titration. 7. Dispense no more than a 90 days' supply for patients on a maintenance dose.

At All Times	**My pharmacy must:** 8. Report adverse events of heart failure due to systolic dysfunction to Cytokinetics, Inc. 9. Not distribute, transfer, loan, or sell MYQORZO, except to a certified pharmacy. 10. Maintain records of dispensing information and provide such data during audits conducted by Cytokinetics, Inc. or a third party acting on behalf of Cytokinetics, Inc. 11. Maintain records of completion of the REMS training by relevant staff. 12. Comply with audits conducted by Cytokinetics, Inc. or a third party acting on behalf of Cytokinetics, Inc. to ensure that all processes and procedures are in place and are being followed.
Maintain Certification to Dispense	**To maintain certification to dispense:** 13. Have any new Authorized Representative enroll by completing and submitting the **Pharmacy Enrollment Form** to the REMS.

Patient Requirements

Only patients enrolled in the MYQORZO REMS can receive MYQORZO.

Patients must:

Be Enrolled	**Before treatment initiation (first dose):** 1. Receive counseling from the healthcare provider on the risk of heart failure due to systolic dysfunction (where the heart cannot pump with enough force) using the **Patient Guide**. 2. Get an echocardiogram to check their heart. 3. Enroll in the REMS by completing the **Patient Enrollment Form** with the healthcare provider. Enrollment information will be provided to the REMS.
Be Monitored	**During treatment, between 2 to 8 weeks after treatment initiation and each dose change, then every 3 or 6 months thereafter (depending on echocardiogram results):** 4. Receive counseling from the healthcare provider on the risk of heart failure due to systolic dysfunction (where the heart cannot pump with enough force) using the **Patient Guide**. 5. Get an echocardiogram to check their heart.
At All Times	6. Inform the healthcare provider or seek other medical attention if there are new or worsening symptoms of heart failure.

Resources

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The resources listed below are available for download at www.MYQORZOREMS.com.

Healthcare Provider

- Prescribing Information
- **REMS Overview**
- **Education Program for Healthcare Providers and Pharmacies**
- **Healthcare Provider Knowledge Assessment**
- **Healthcare Provider Enrollment Form**
- **Patient Guide**
- **Patient Enrollment Form**
- **Patient Monitoring Form**

Pharmacy

- Prescribing Information
- **REMS Overview**
- **Education Program for Healthcare Providers and Pharmacies**
- **Pharmacy Enrollment Form**

Patient

- Medication Guide
- **Patient Guide**
- **Patient Enrollment Form**

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For more information about the MYQORZO REMS, visit www.MYQORZOREMS.com or call the MYQORZO REMS at 1-844-285-7367. For MYQORZO REMS Call Center hours, visit www.MYQORZOREMS.com.

Please see Important Safety Information, including BOXED WARNING, and US Full Prescribing Information for MYQORZO here: cytokinetics.com/pi_myqorzo.pdf.

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MYQORZO™ REMS

Healthcare Provider Knowledge Assessment

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Instructions:

1. Complete and submit this **Healthcare Provider Knowledge Assessment** to the REMS:
   - Online at **www.MYQORZOREMS.com**, or
   - Print and complete this form and submit by fax to 1-844-285-7399

   A passing score of 100% is needed to successfully pass the knowledge assessment.
2. To complete certification, enroll by completing and submitting the **Healthcare Provider Enrollment Form**.

You will be notified of successful certification within 2 business days once the **Healthcare Provider Enrollment Form** has also been submitted.

Healthcare Provider Information (Fields marked * are REQUIRED)		
*First Name:	Middle Initial:	*Last Name:
*Healthcare Provider NPI #:		
*Phone Number: Area Code/Telephone Number	*Fax Number: Area Code/Fax Number	
*Email:		

For questions 1–10, select the correct answer. You must answer ALL questions correctly to become certified in the REMS.

Question 1: The risk being mitigated by the MYQORZO REMS is:

- ☐ A. Liver function test elevations (>3 times the upper limit of normal)
- ☐ B. Neutropenia
- ☐ C. Heart failure due to systolic dysfunction
- ☐ D. Severe skin reactions

Question 2: Which of the following is an objective of the MYQORZO REMS?

- ☐ A. Monitoring for detection of heart failure due to systolic dysfunction with routine echocardiograms
- ☐ B. Screening for neutropenia prior to and during treatment with MYQORZO
- ☐ C. Both A & B
- ☐ D. None of the above

Question 3: Initiation of MYQORZO in patients with LVEF <55% is not recommended.

- ☐ A. True
- ☐ B. False

Question 4: What is the recommended starting dose of MYQORZO?

- ☐ A. 5 mg once daily
- ☐ B. 10 mg once daily
- ☐ C. 15 mg once daily

MYQORZO™ REMS

Healthcare Provider Knowledge Assessment

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Question 5: Before treatment initiation with MYQORZO, you must counsel the patient, using the **Patient Guide**, on:

- ☐ A. The risk of heart failure due to systolic dysfunction
- ☐ B. The need to screen patients for severe skin reaction
- ☐ C. All of the above

Question 6: Echocardiograms are required at each of the following time points while on MYQORZO, **except:**

- ☐ A. 2 to 8 weeks after initiation of treatment
- ☐ B. 2 to 8 weeks after dose adjustment
- ☐ C. Every 3 months in patients with LVEF <55% and ≥50%
- ☐ D. Once a year starting in the first year

Question 7: **Patient Monitoring Forms**, confirming the patient's cardiovascular status has been assessed by echocardiogram, must be received by the REMS:

- ☐ A. Within 60 days of initiating the first dose
- ☐ B. Within 60 days following any dose change
- ☐ C. After the maintenance dose has been established: every 6 months (in patients with LVEF ≥55%) or every 3 months (in patients with LVEF <55% and ≥50%) from the most recent **Patient Monitoring Form**
- ☐ D. All of the above

Question 8: Patients do not need to enroll in the REMS to receive treatment with MYQORZO.

- ☐ A. True
- ☐ B. False

Question 9: A prescription for MYQORZO must be sent to a certified pharmacy to be dispensed.

- ☐ A. True
- ☐ B. False

Question 10: If LVEF is <50% and ≥40%, you should:

- ☐ A. Decrease the dose if patient is on 10 mg, 15 mg or 20 mg dose
- ☐ B. Interrupt treatment for 7 days if the patient is on 5 mg dose
- ☐ C. Maintain current dose and conduct an echocardiogram in 6 months
- ☐ D. Both A & B

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MYQORZO™ REMS

Patient Guide

What Is MYQORZO (aficamten)?

MYQORZO is a prescription medicine used to treat adults with symptomatic obstructive hypertrophic cardiomyopathy (oHCM).

What Is the Most Serious Risk of Treatment With MYQORZO?

In some patients, MYQORZO can cause heart failure, which is where the heart cannot pump with enough force.

- Heart failure is a serious condition that can lead to death.
- People may have a greater risk of heart failure during treatment with MYQORZO if they develop:
  - A serious infection, or
  - A new or worsening heart rhythm problem.

What Is the MYQORZO REMS?

A **R**isk **E**valuation and **M**itigation **S**trategy (REMS) is a drug safety program that the U.S. Food and Drug Administration (FDA) can require for certain medicines to ensure they are used safely. Drug companies and healthcare providers must take extra steps to make sure the benefits of using the drug are greater than the risks. FDA must approve these steps as part of the REMS.

MYQORZO has a REMS because of the risk of heart failure due to systolic dysfunction.

What Should I Know About Treatment With MYQORZO?

To receive MYQORZO, you must have tests called echocardiograms on a regular basis so your healthcare provider can see how your heart is responding to MYQORZO. Your healthcare provider will order these tests for you.

When you are taking MYQORZO, tell your healthcare provider or get medical help right away if you develop any of the following new or worsening symptoms:

- Shortness of breath
- Swelling in your legs
- Fatigue
- Chest pain

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What Do I Need to Do Before Starting MYQORZO?

BEFORE TAKING MYQORZO (FIRST DOSE)

1. Receive counseling from your healthcare provider on the risk of heart failure due to systolic dysfunction (where the heart cannot pump with enough force) using this **Patient Guide**. Discuss with your healthcare provider and understand the following:
   - Risk of heart failure due to systolic dysfunction, which is where your heart cannot pump with enough force.
   - New or worsening symptoms of heart failure.
   - Need to report new or worsening symptoms of heart failure to your healthcare provider or to seek other medical attention.
   - Need to get echocardiograms before starting treatment with MYQORZO and routinely during treatment based on the MYQORZO Prescribing Information.
   - Especially tell your healthcare provider if you:
     - Develop a severe infection.
     - Develop a new or worsening heart rhythm problem.
2. Get an echocardiogram to check your heart and see if you are eligible for treatment with MYQORZO.
3. Enroll in the REMS by completing the **Patient Enrollment Form** with your healthcare provider. Enrollment information will be provided to the REMS.

What Do I Need to Do While Taking MYQORZO?

WHILE TAKING MYQORZO

4. Discuss with your healthcare provider and, using this **Patient Guide**, understand the:
   - Risk of heart failure due to systolic dysfunction, where your heart cannot pump with enough force.
   - New or worsening symptoms of heart failure.
   - Need to report new or worsening symptoms of heart failure to your healthcare provider or to seek other medical attention.
5. Get an echocardiogram to check your heart and see if you are eligible to continue treatment with MYQORZO (2 to 8 weeks after treatment initiation and each dose change, and then every 3 or 6 months thereafter, depending on echocardiogram results).

AT ALL TIMES

6. Inform your healthcare provider or seek other medical attention if there are new or worsening symptoms of heart failure.

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Where Will I Get My MYQORZO?

MYQORZO is only dispensed by pharmacies that are certified to participate in the MYQORZO REMS.

The pharmacy will contact you to fill your prescription and ship it to your home.

Where Can I Get More Information on MYQORZO?

You can find additional information at www.MYQORZOREMS.com or call the MYQORZO REMS at 1-844-285-7367.

- For MYQORZO REMS Call Center hours, visit www.MYQORZOREMS.com

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MYQORZO™ REMS

Patient Monitoring Form

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This completed form must be reviewed and submitted by a certified healthcare provider or their delegate.

Instructions:

Submit this form to the REMS either:

- Online at **www.MYQORZOREMS.com**, or
- By faxing the completed form to the MYQORZO REMS to **1-844-285-7367**

This form must be submitted in its entirety on the appropriate REMS schedule for your patient to receive MYQORZO. Failure to submit a Patient Monitoring Form may result in delay in patient access to MYQORZO.

After a patient has started taking MYQORZO, echocardiograms must be done:



2-8 weeks after initiating treatment



2-8 weeks after dose adjustment



Every 3 or 6 months thereafter (depending on echocardigram results)

After completing the echocardiogram, this form should be submitted per the following schedule:

- Treatment Initiation:
  - Within 60 days of initiating the first dose
- Titration / Dose Change:
  - Within 60 days of the dose change
- Maintenance Dose:
  - For patients with LVEF ≥55% every 6 months from the most recent **Patient Monitoring Form**
  - For patients with LVEF <55% and ≥50% every 3 months from the most recent **Patient Monitoring Form**

Important: While you have up to the timeframes listed above, please submit this form as soon as possible after completing the echocardiogram.

Note: MYQORZO REMS support staff may fill out this form on behalf of the certified healthcare provider of record. The certified healthcare provider or their delegate is responsible for reviewing and submitting this form, as well as ensuring compliance with the REMS requirements; this includes overseeing the monitoring, evaluation, and management of each patient under their care.

Healthcare Provider Information (Fields marked * are REQUIRED)	
*Healthcare Provider First Name:	*Last Name:
*Healthcare Provider NPI #:	*Phone:
*Fax:	*Email:

Reference ID: 5715575

MYQORZO™ REMS

Patient Monitoring Form

MYQORZO™ (aficamten) 5·10·15·20 mg tablets

Patient Information

*Patient First Name:	Middle Initial:	*Last Name:
*Birthdate (MM/DD/YYYY):	*Phone:	

Patient Monitoring

***Is the above-named patient still under your care?**

☐ Yes (If **yes**, answer questions 1-5 below) ☐ No (If **no**, complete Patient Transition of Care section below†)

1. *I have reviewed the patient's echocardiogram results in accordance with the Prescribing Information.

☐ Yes ☐ No (If **no**, the patient cannot receive MYQORZO)

2. *What is the patient's LVEF?

☐ a. ≥55%

☐ b. <55% and ≥50%

☐ c. <50% and ≥40% (decrease dose by 5 mg if at 20 mg, 15 mg or 10 mg; interrupt treatment for 7 days if at 5 mg)

☐ d. <40% (treatment should be interrupted for at least 7 days; a new **Patient Monitoring Form** documenting LVEF ≥55% is required to resume treatment at 5 mg)

3. *Has the patient experienced a clinical heart failure event requiring clinical intervention or hospitalization since the last submitted form?

☐ Yes ☐ No

4. *Is the patient authorized to continue treatment?

☐ Yes ☐ No

5. *After this clinical visit, what will be the patient's next MYQORZO dose?

☐ 5 mg ☐ 10 mg ☐ 15 mg ☐ 20 mg or ☐ 0 mg (dose interruption)

Healthcare Provider Agreement

I acknowledge that I have counseled the patient on the risk of heart failure due to systolic dysfunction using the **Patient Guide** and assessed the patient's cardiovascular status and the appropriateness of continuing treatment.

***Certified Healthcare Provider Signature:**	***Date (MM/DD/YYYY):**

†Patient Transition of Care (Only complete this section if patient is no longer under your care)

If known, please indicate the name of the healthcare provider now responsible for this patient's care:

Healthcare Provider Name: _____

Healthcare Provider Phone Number: _____

☐ Patient's Current Healthcare Provider Is Unknown

Report all adverse events to Cytokinetics, Inc. at 1-833-633-2986 or the FDA at 1-800-FDA-1088 (1-800-332-1088) or www.FDA.gov/medwatch.

If you have any questions about the MYQORZO REMS, please call the MYQORZO REMS Call Center at 1-844-285-7367. For MYQORZO REMS Call Center hours, visit www.MYQORZOREMS.com.

PHONE: 1-844-285-7367 FAX: 1-844-285-7399 www.MYQORZOREMS.com

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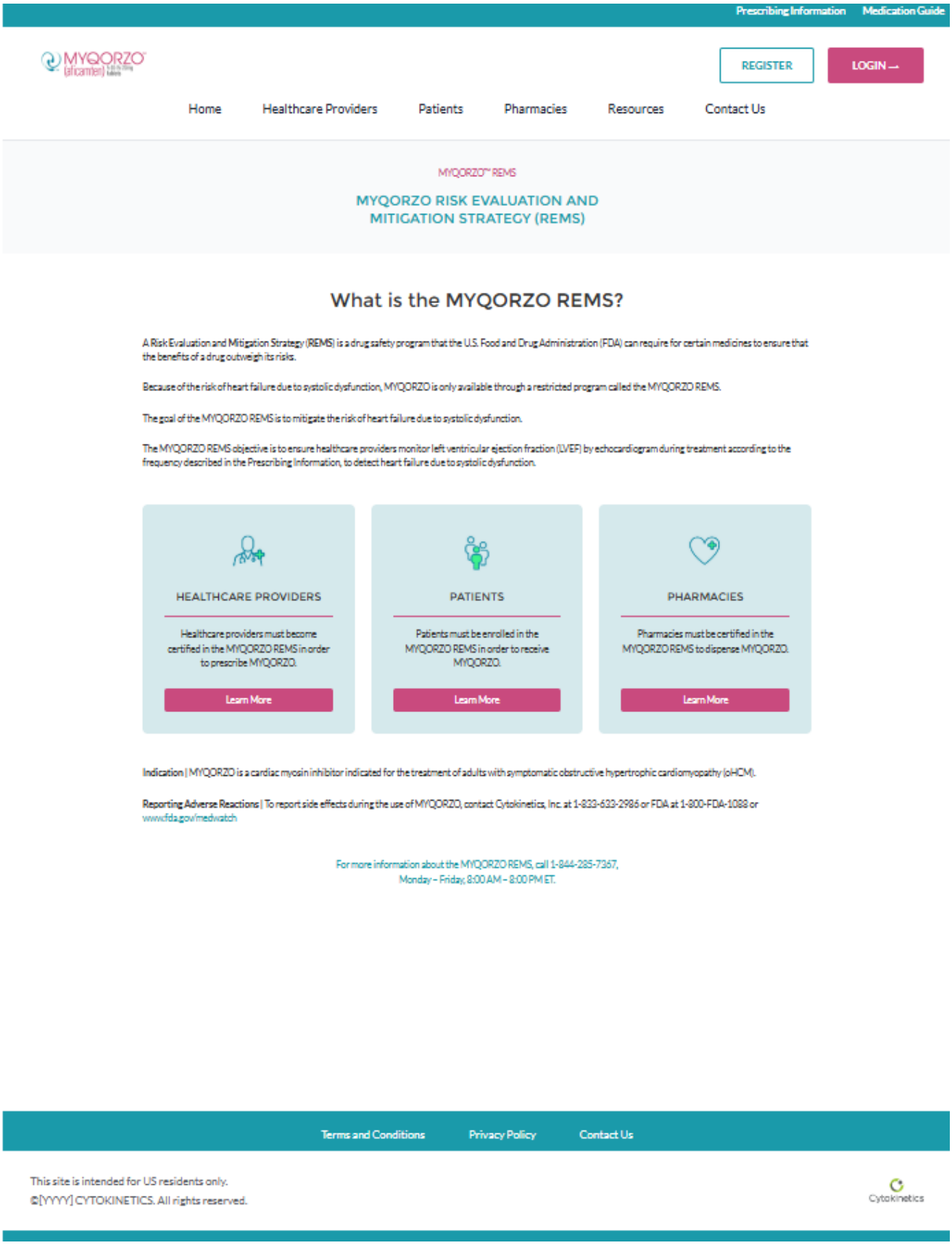
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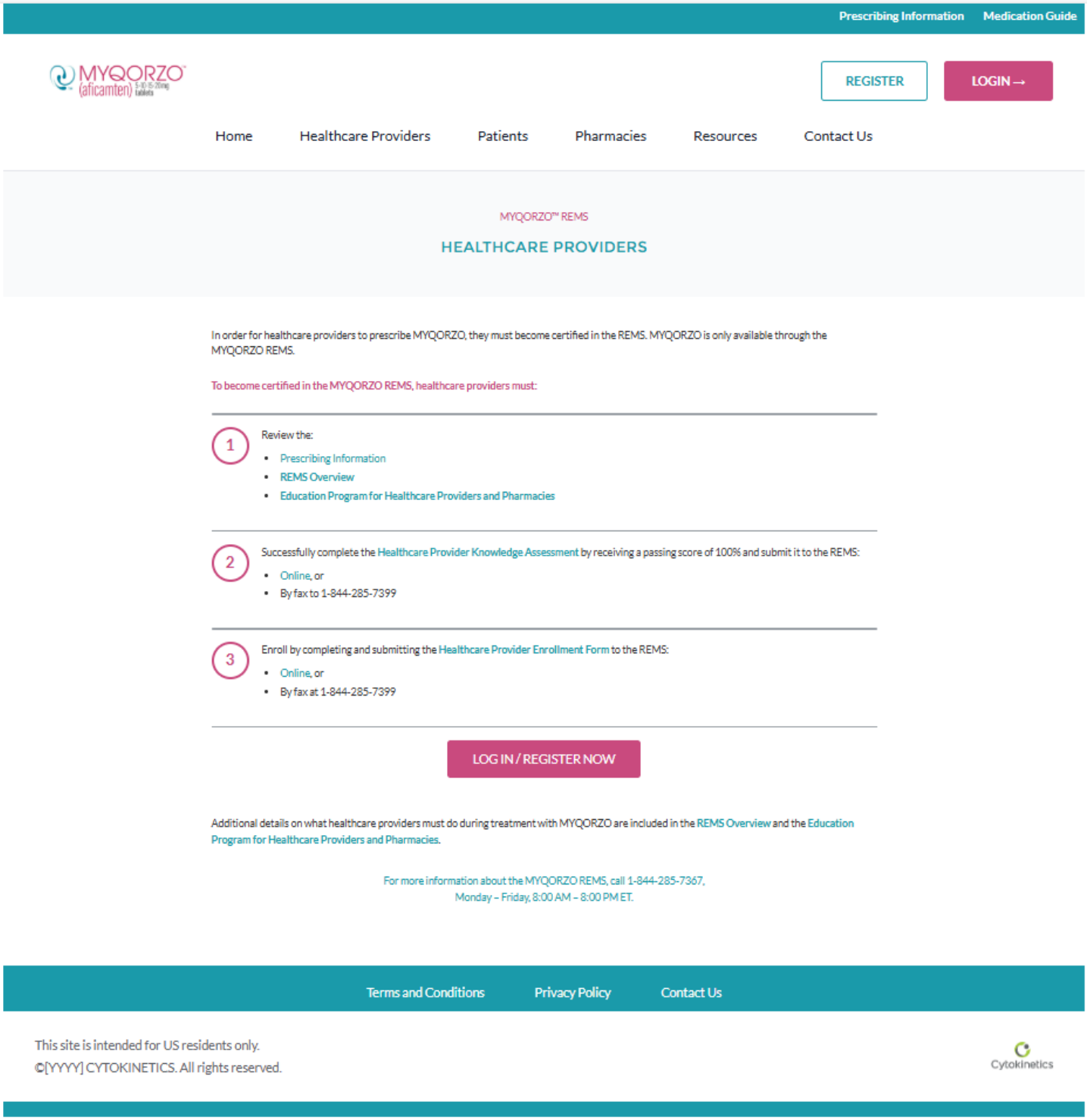
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1. Home / Landing Page



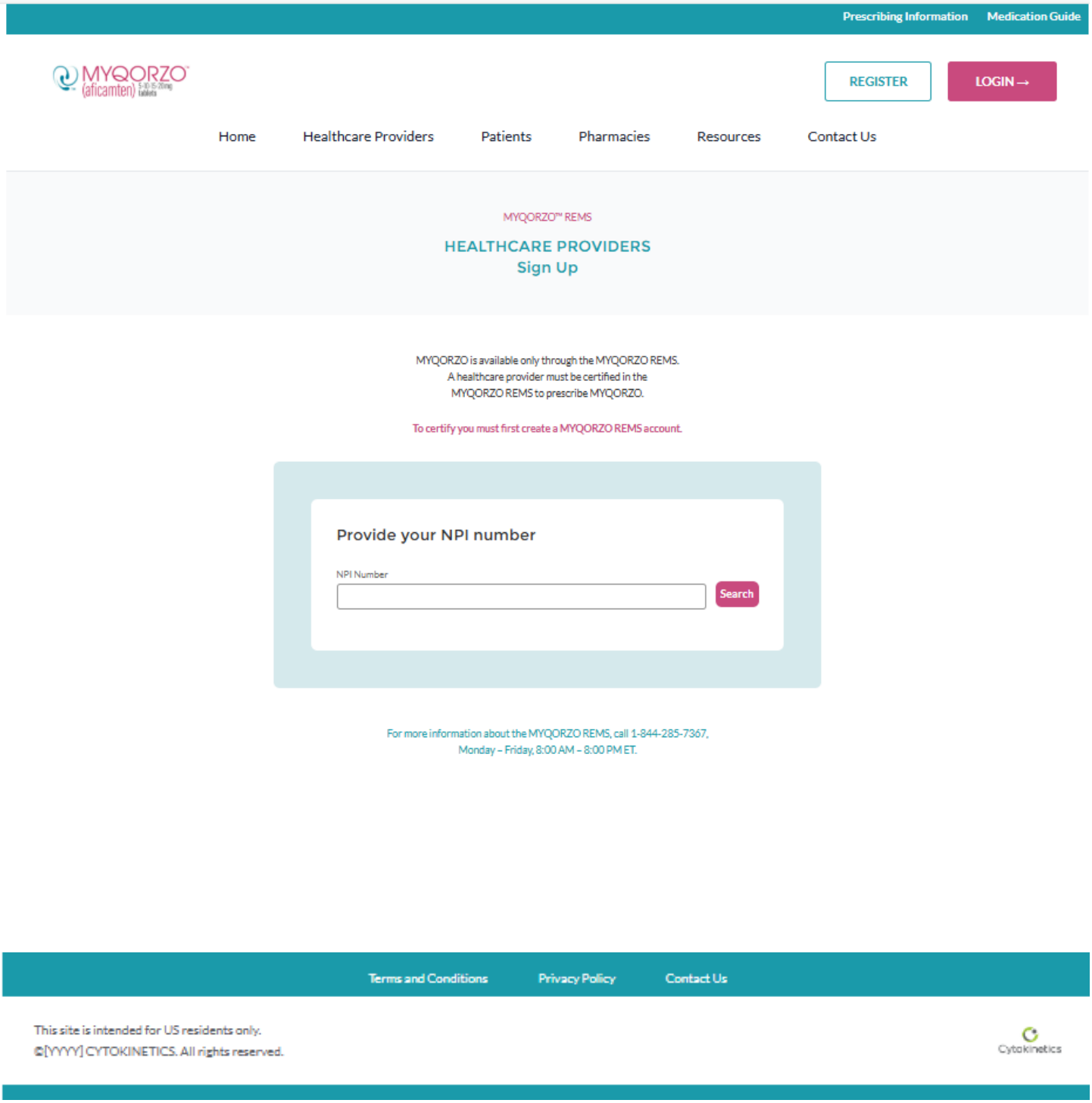
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2. Healthcare Providers Landing Page



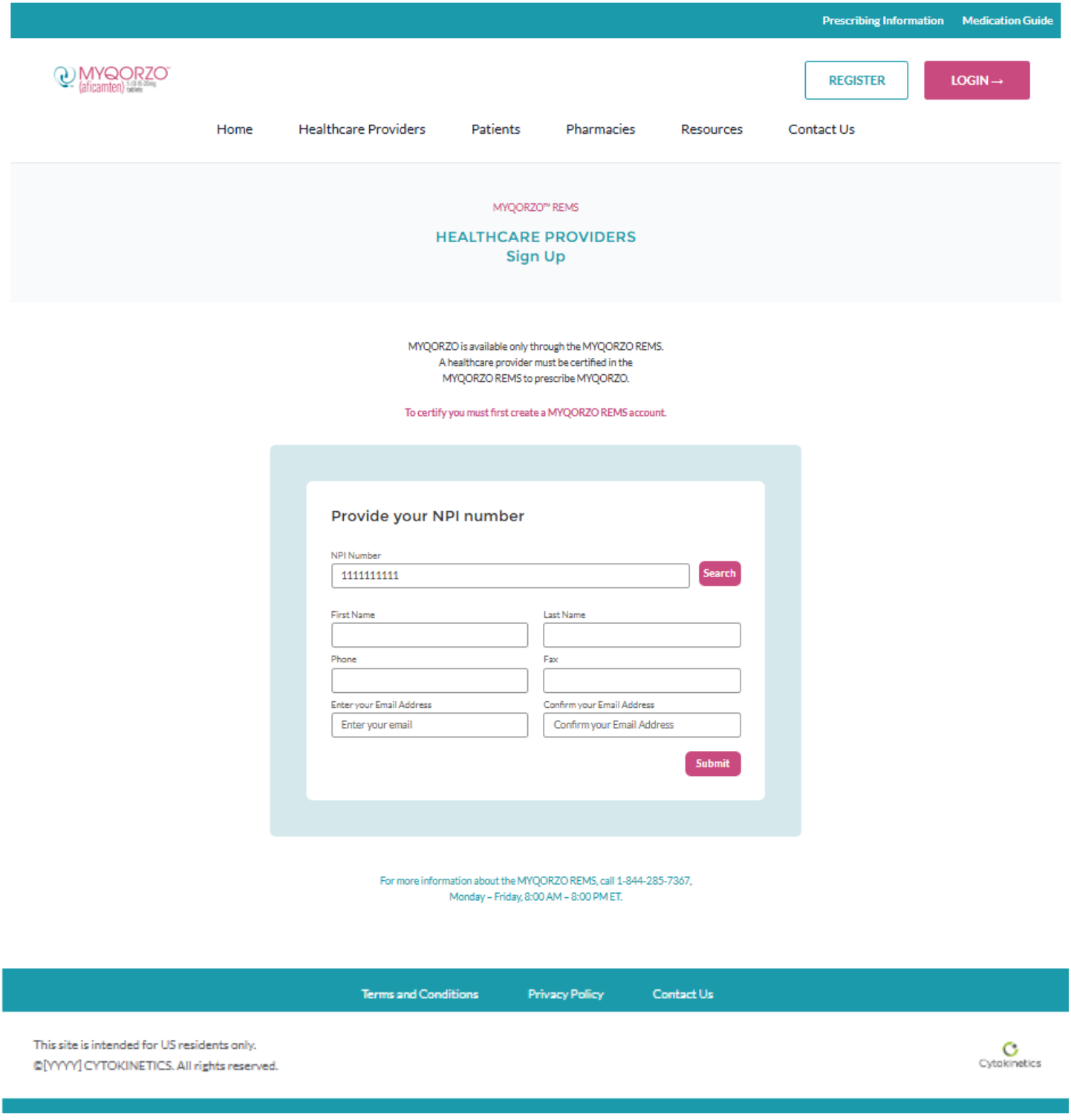
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3. Healthcare Provider — Sign Up (Step 1)



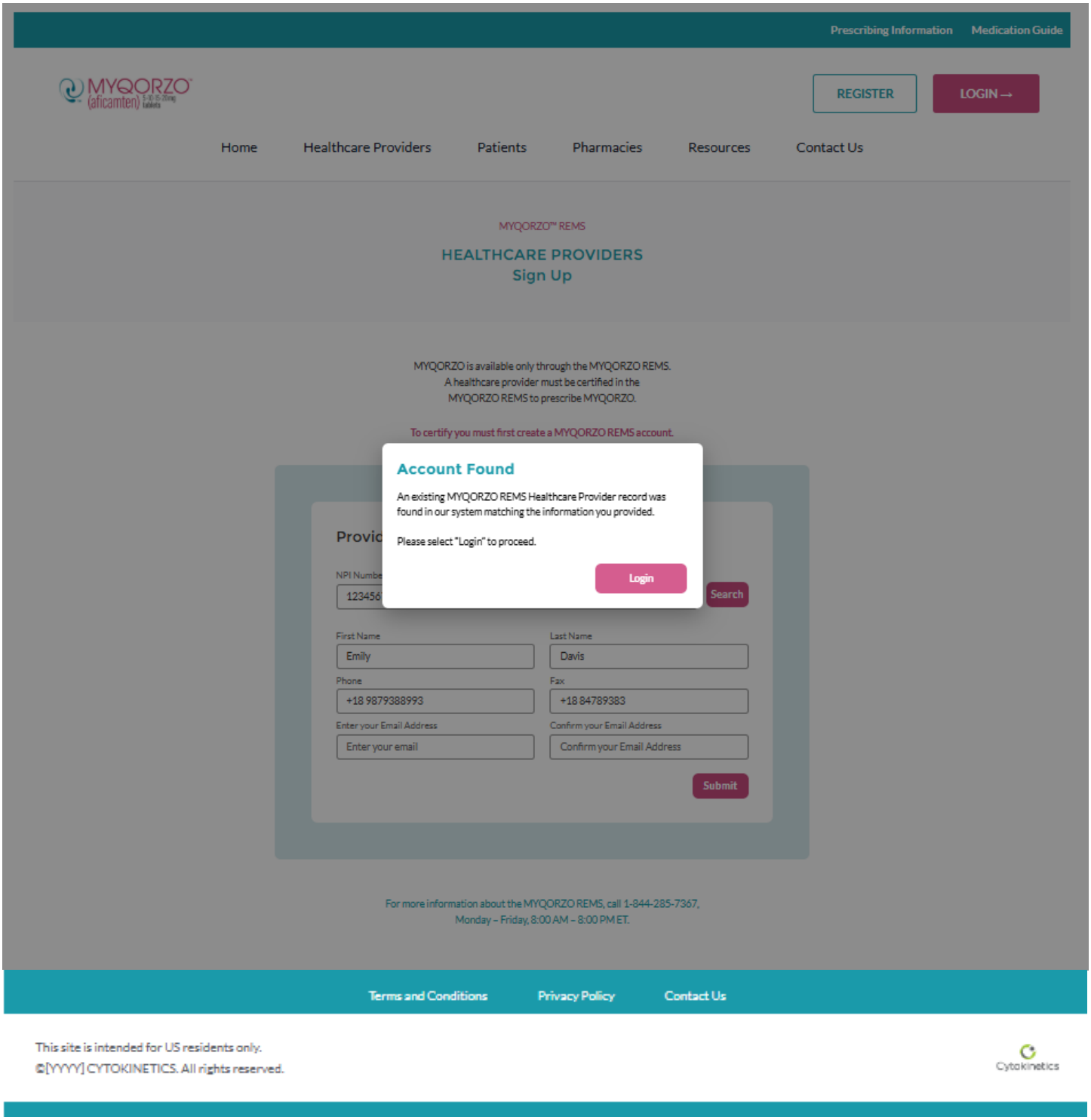
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4. Healthcare Provider — Sign Up (Step 2)



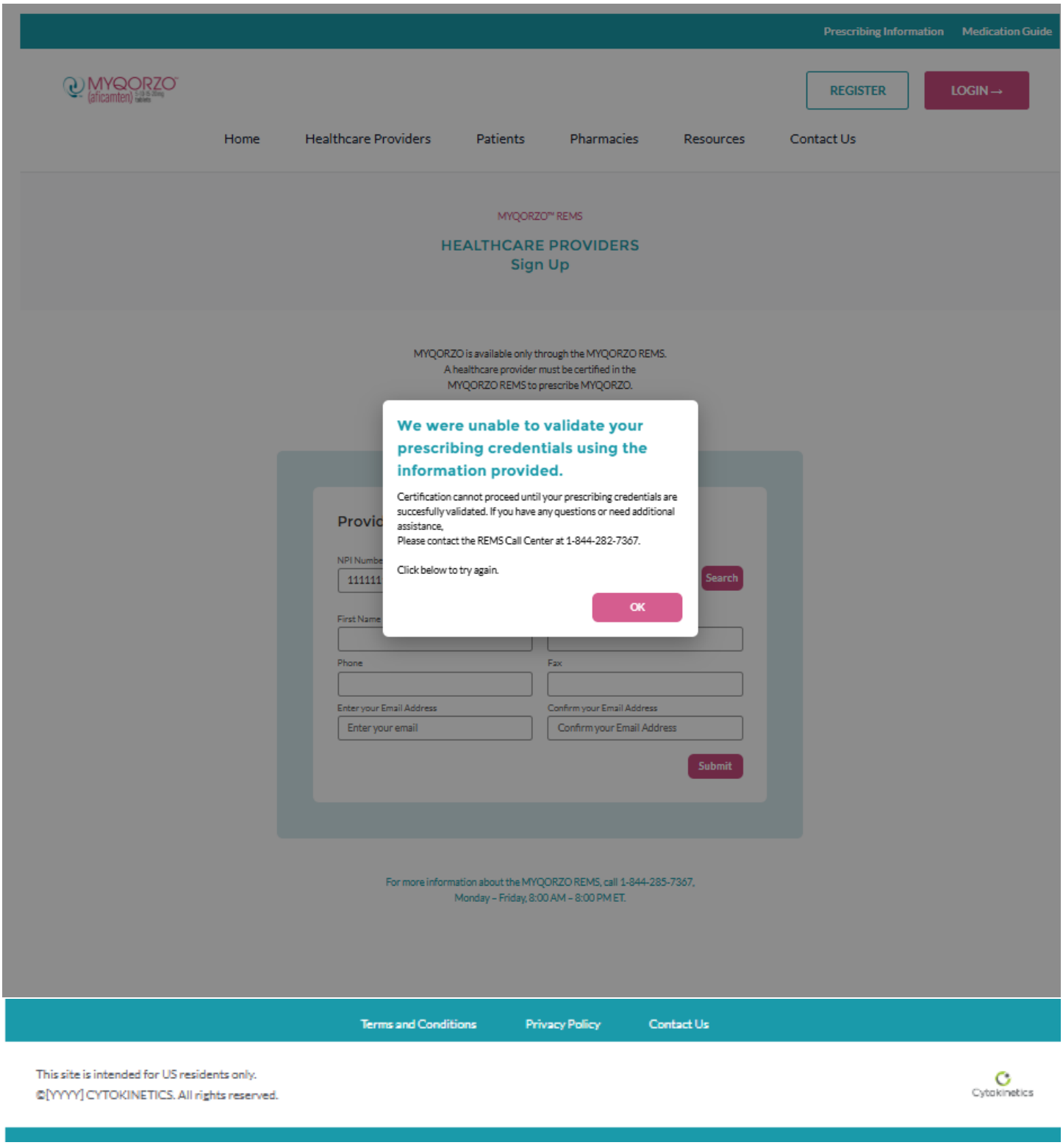
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5. Healthcare Provider — Account Found



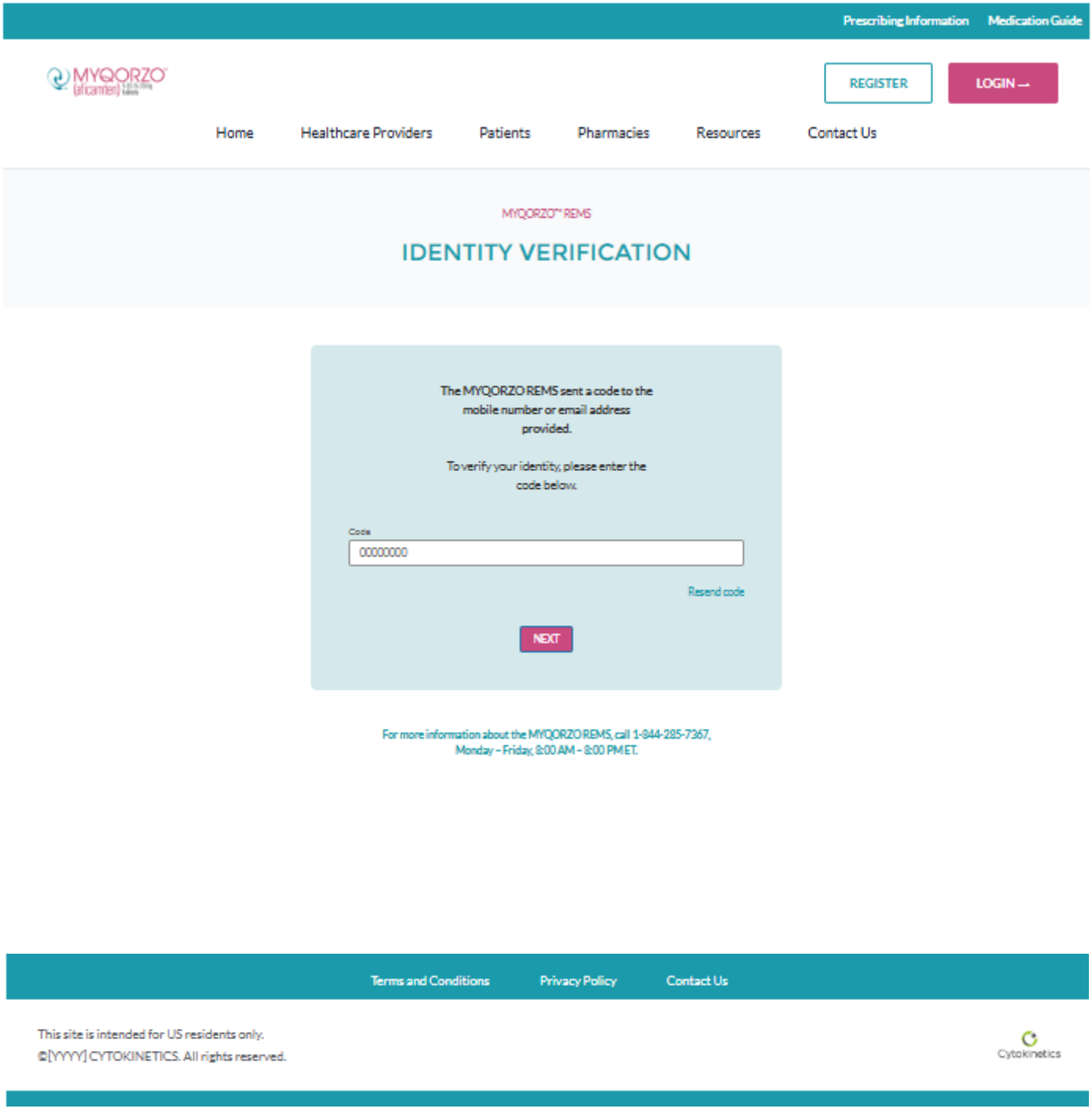
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6. Healthcare Provider — Unsuccessful NPI Verification



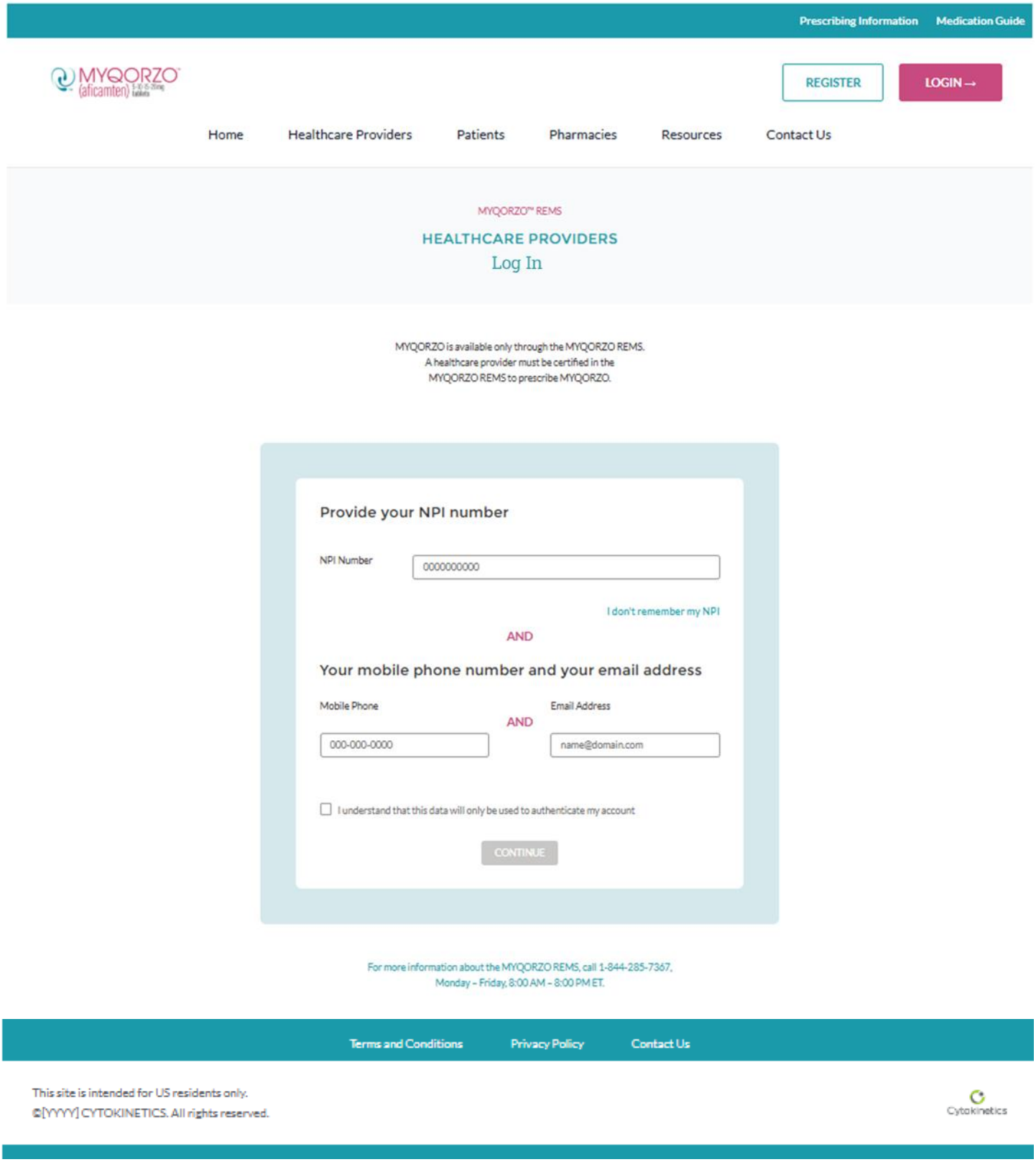
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7. Healthcare Provider — Identity Verification



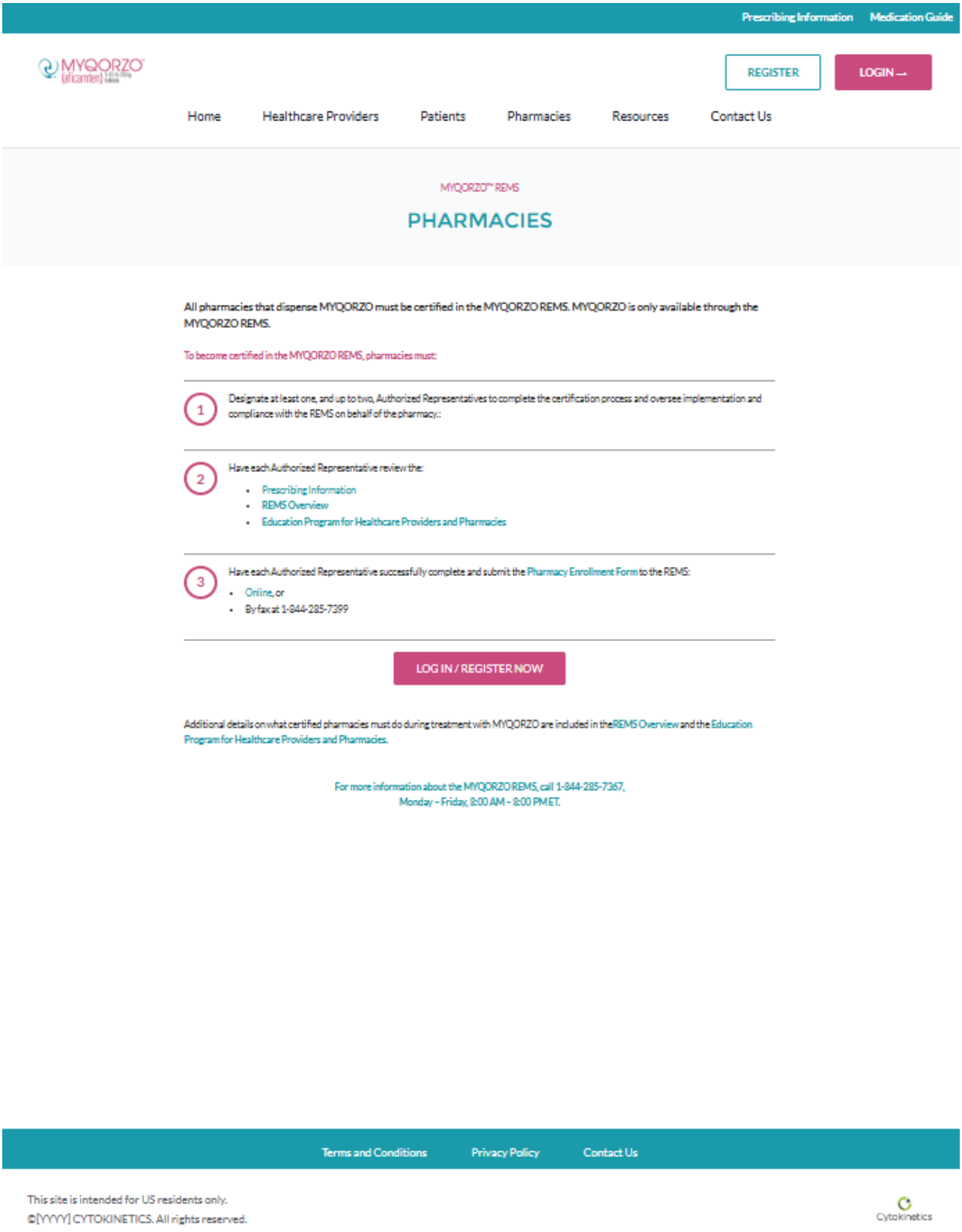
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8. Healthcare Provider — Log In



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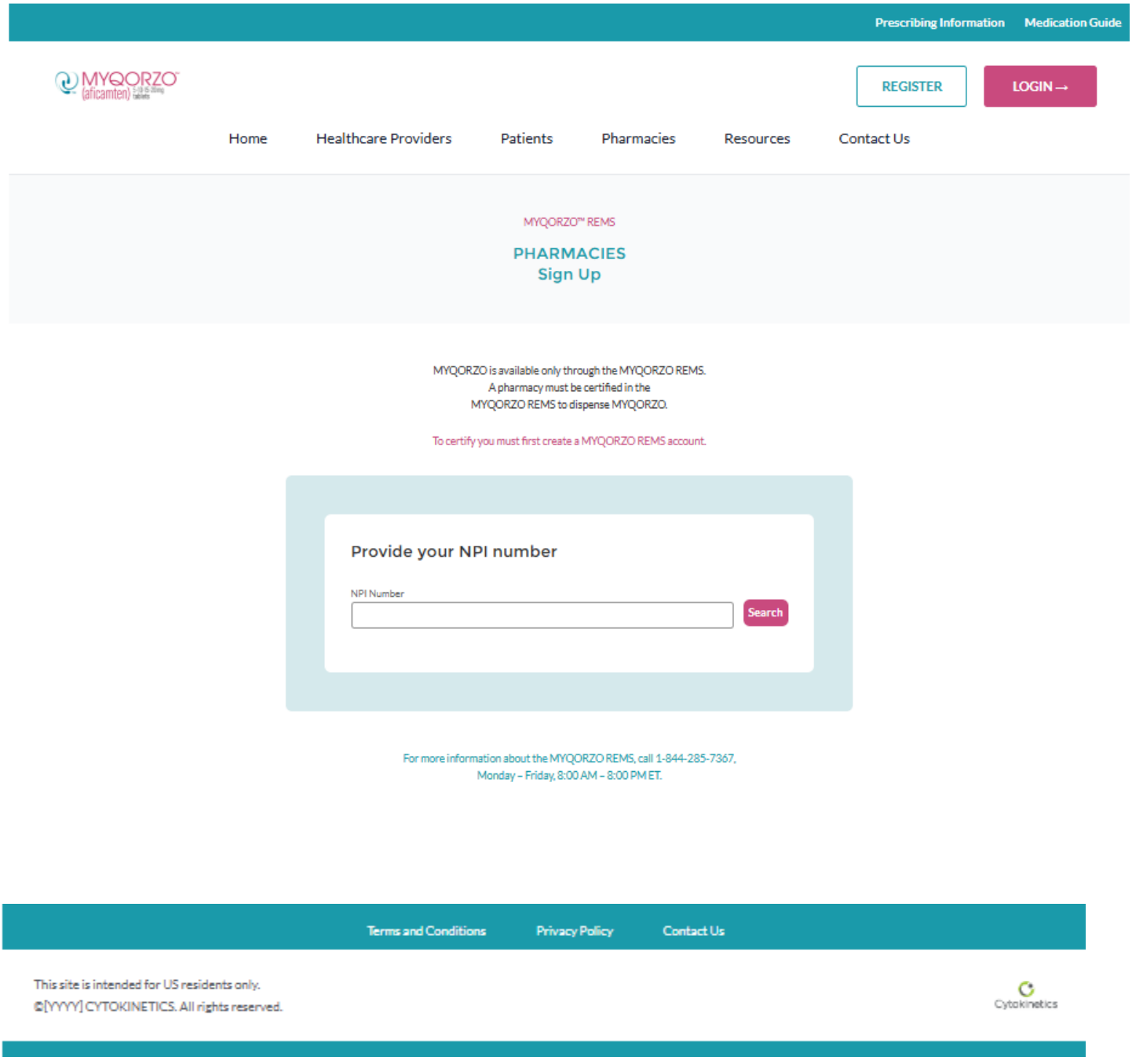
9. Pharmacies Landing Page



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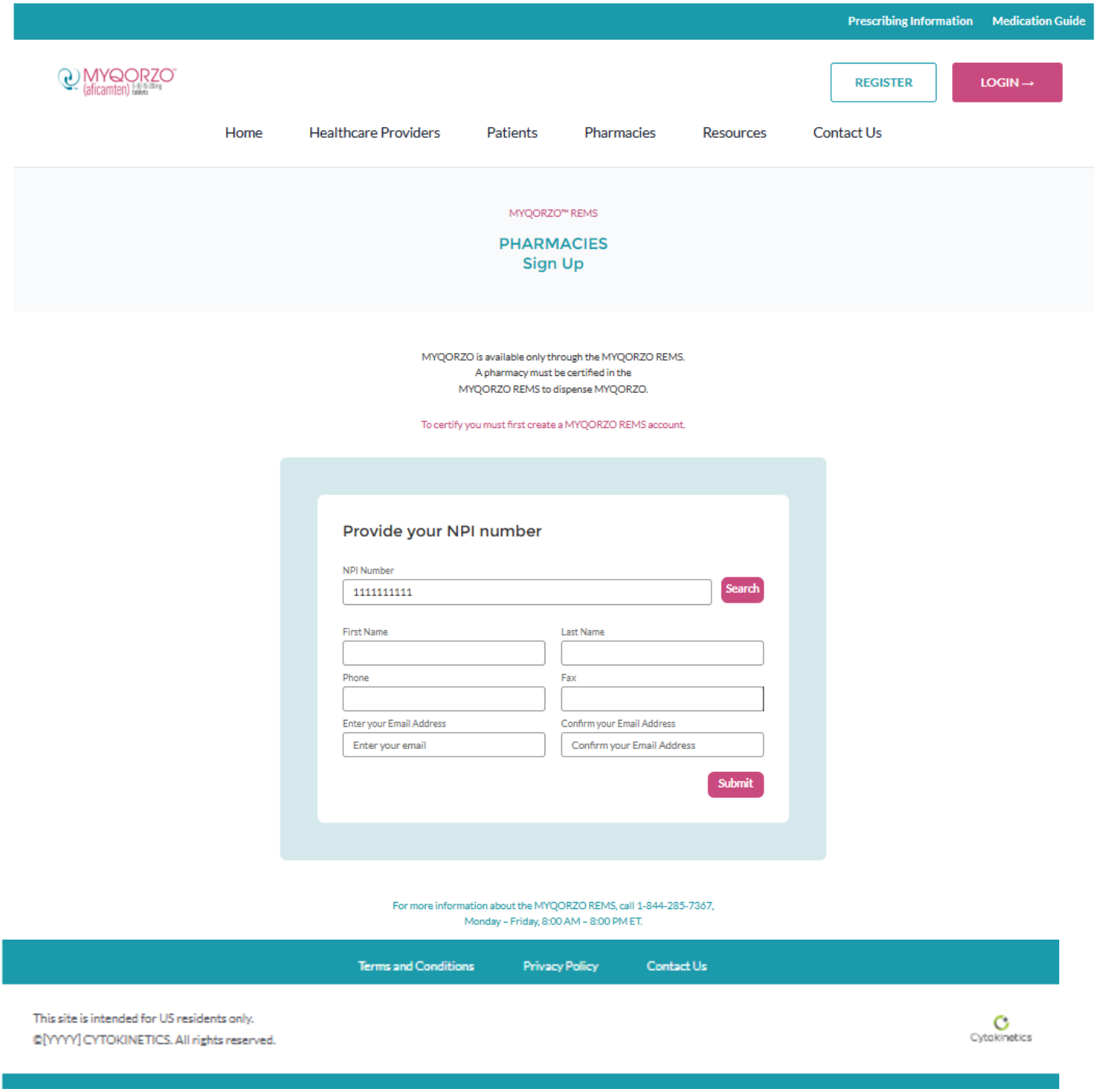
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10. Pharmacy — Sign Up (Step 1)



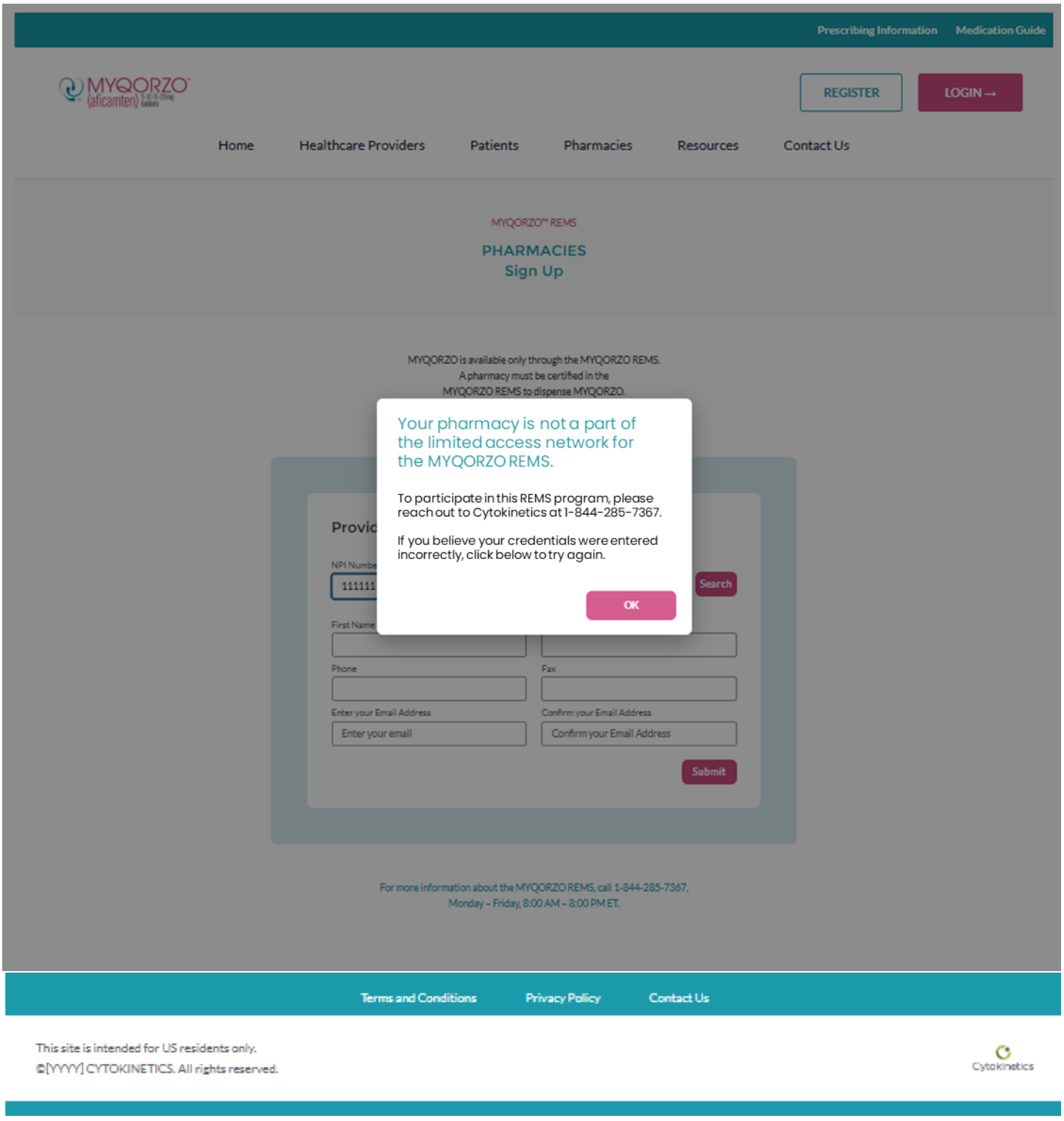
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11. Pharmacy — Sign Up (Step 2)



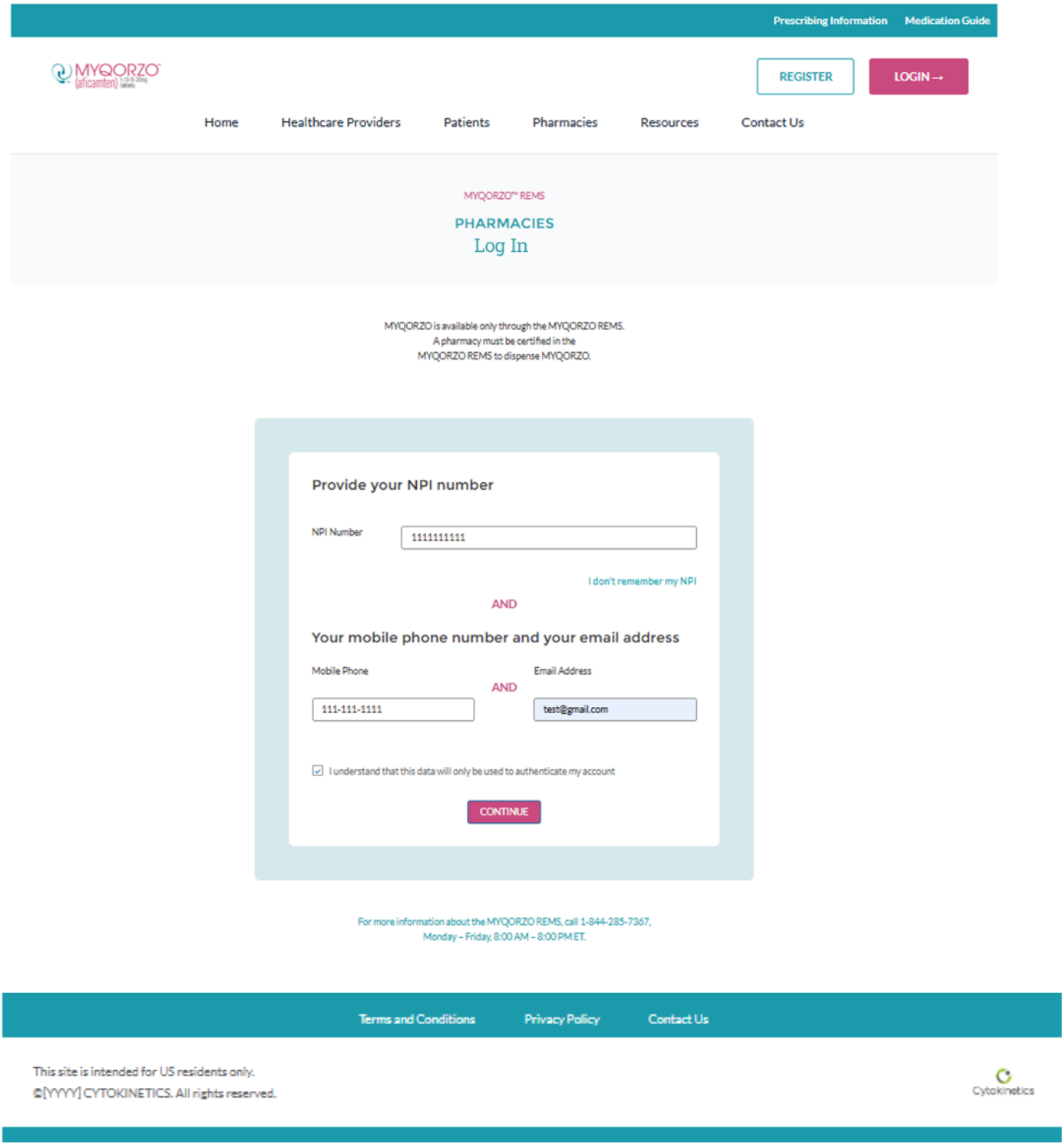
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12. Pharmacy — Unsuccessful NPI Verification



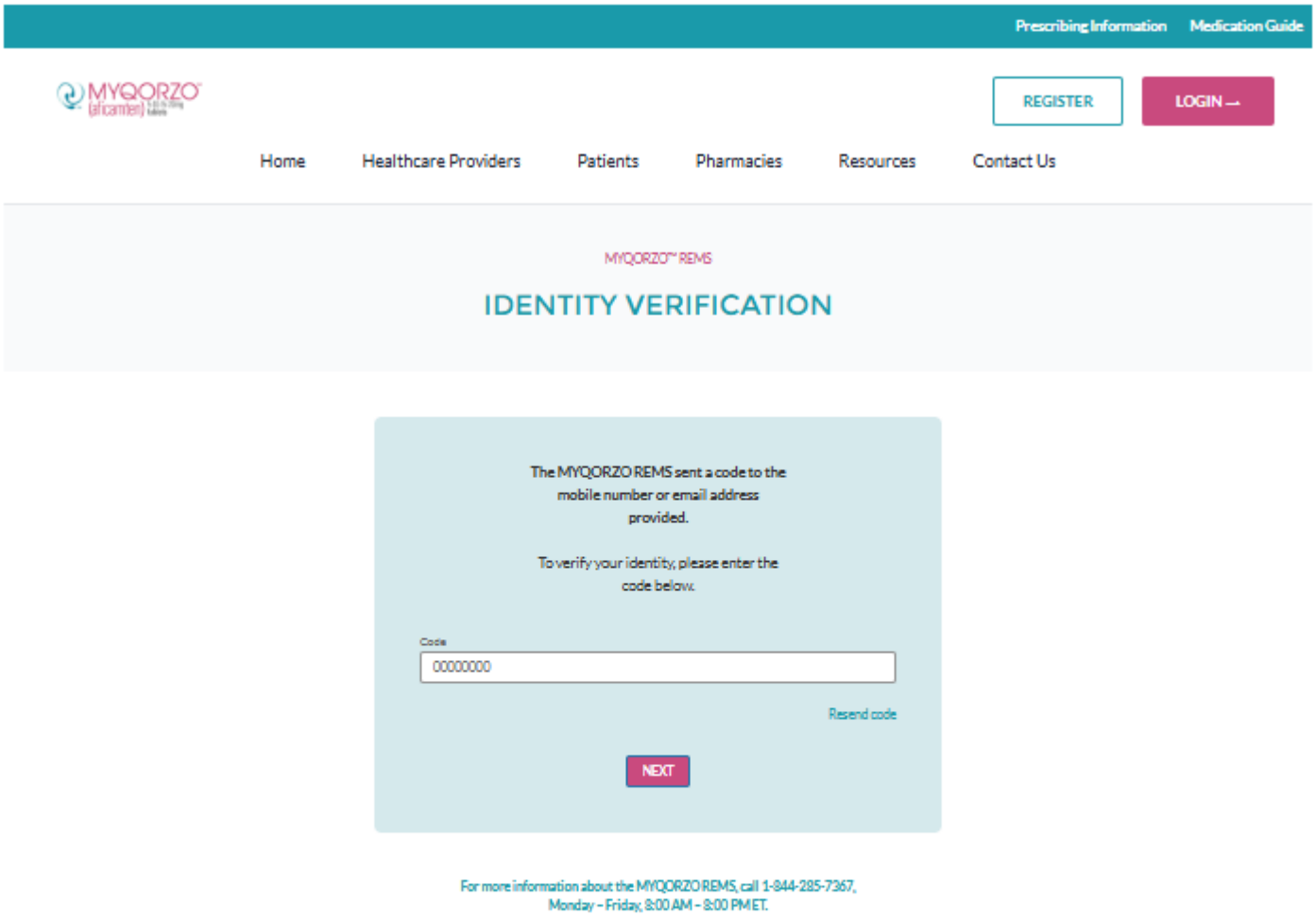
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13. Pharmacy — Log In



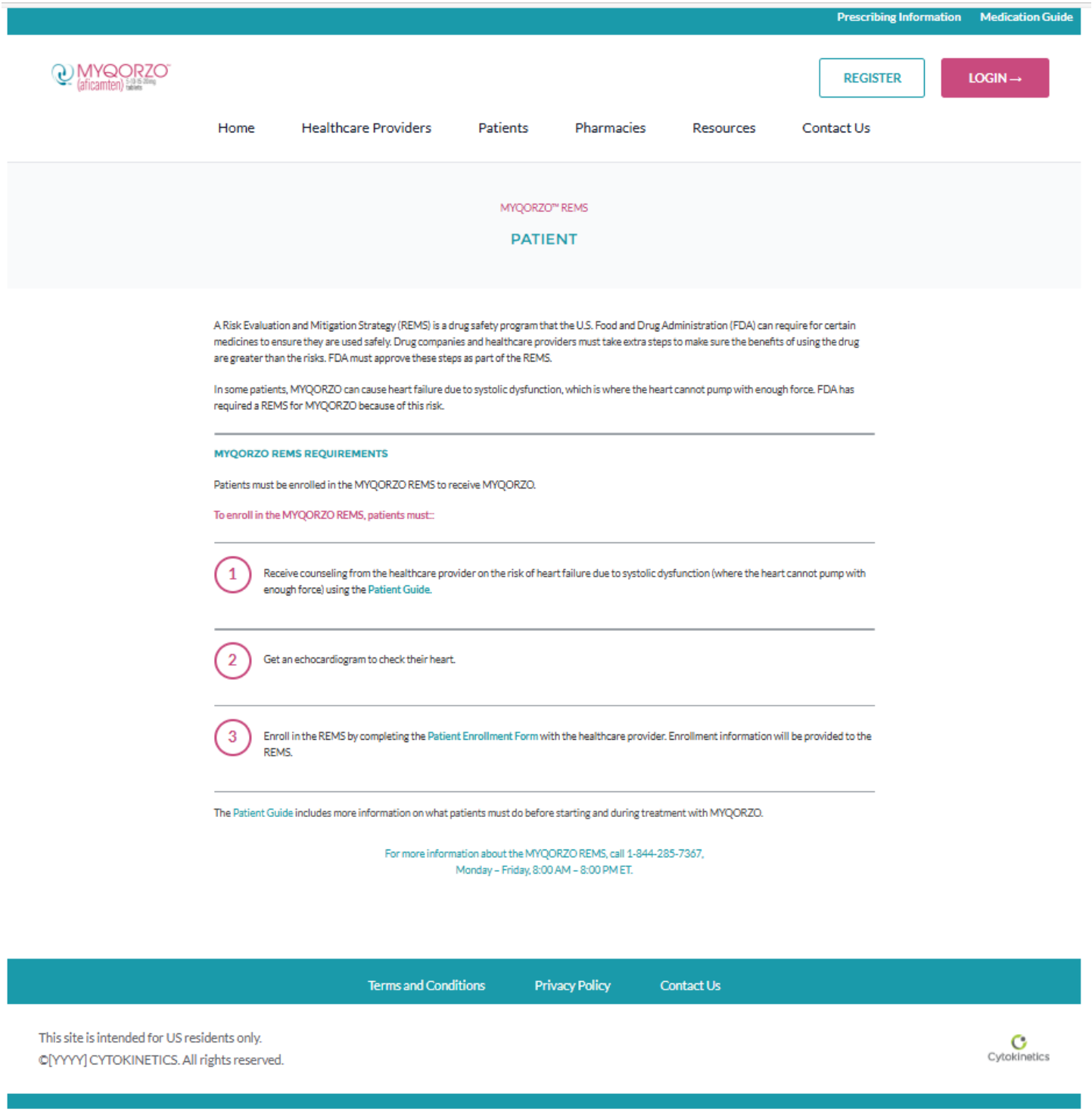
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14. Pharmacy — Identity Verification



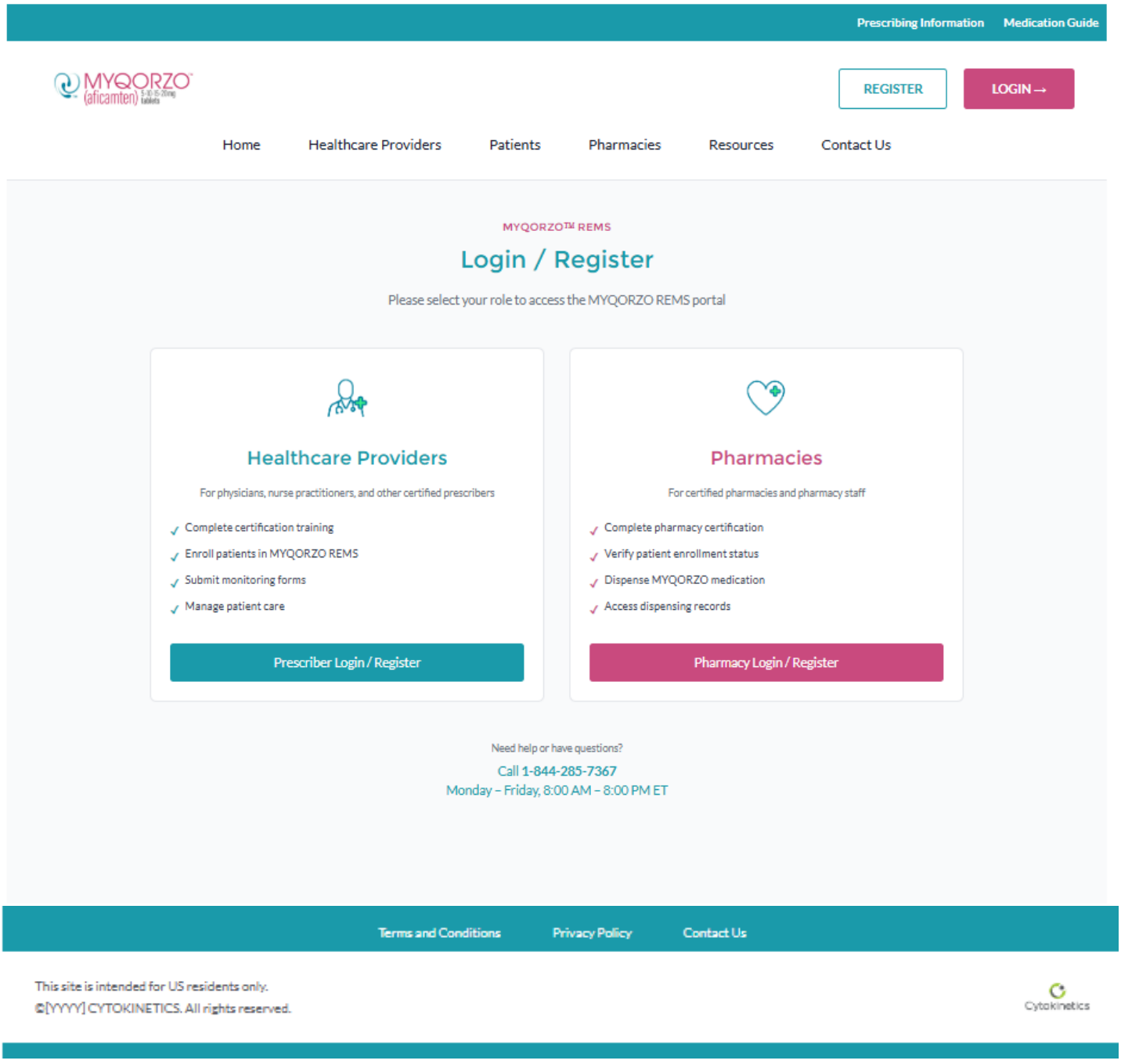
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15. Patients Landing Page



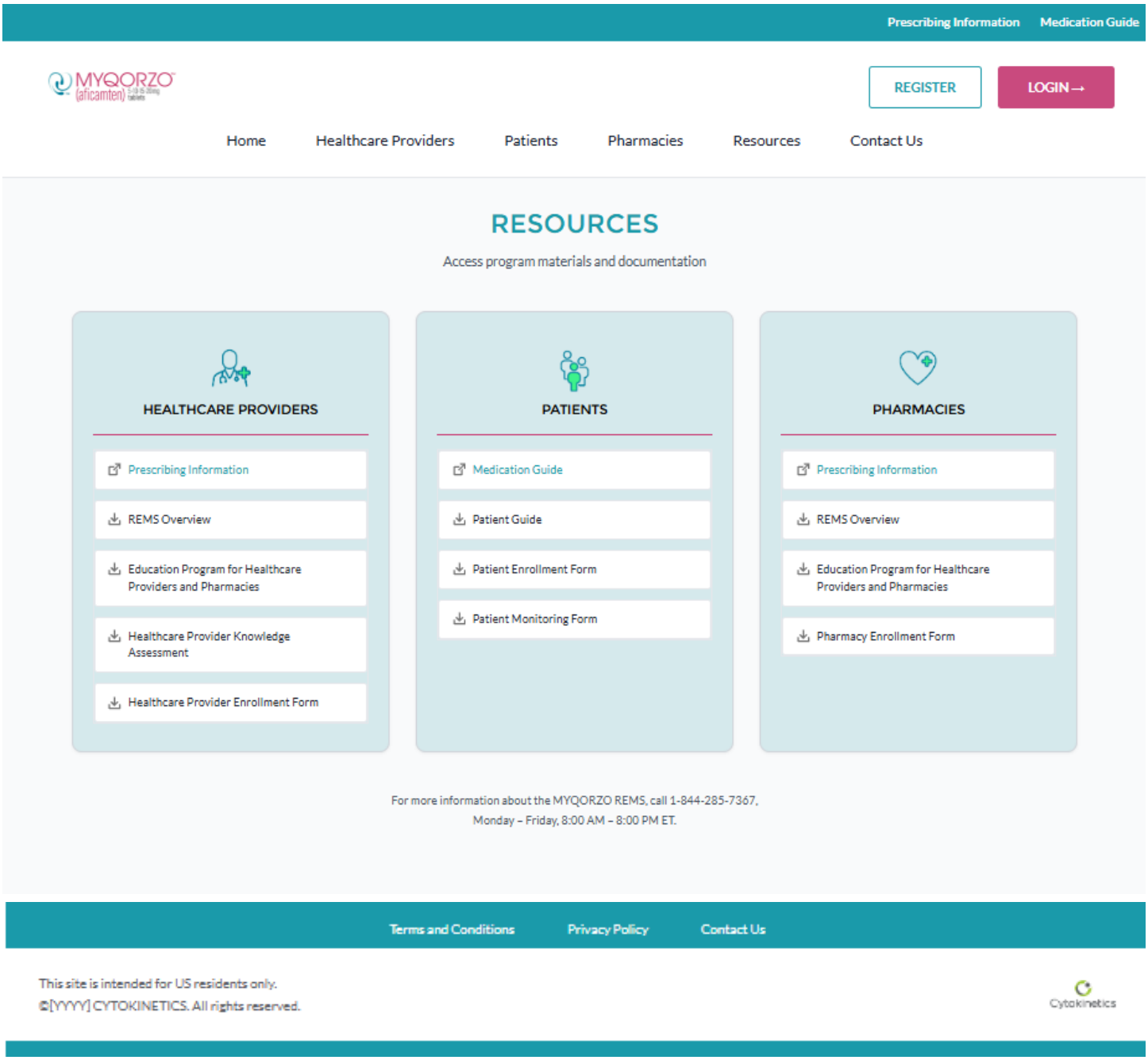
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16. Log In / Register



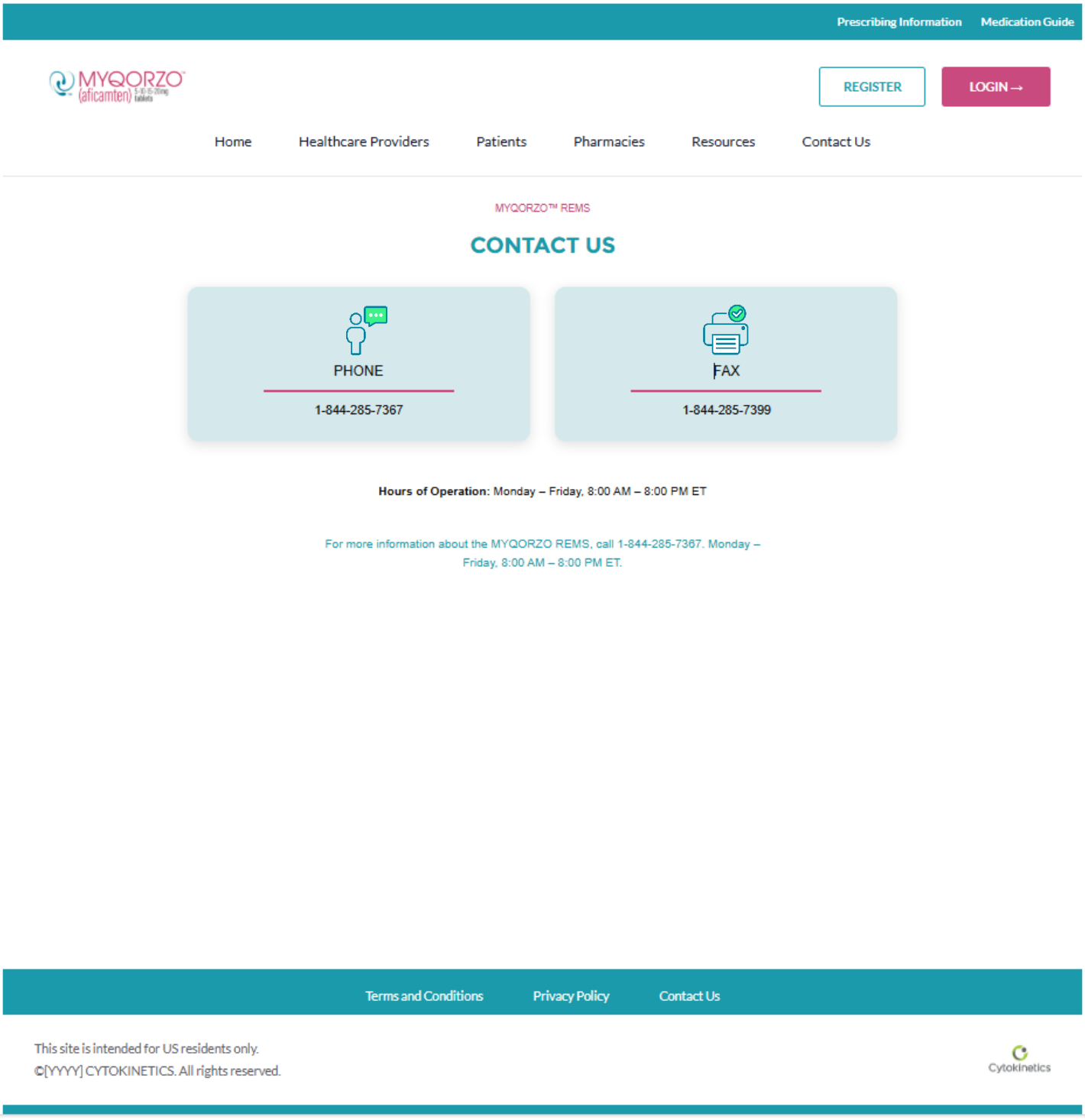
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17. Resources



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18. Contact Us



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